

ARMY PUBLIC SCHOOL DELHI CANTT

2025-26

PSYCHOLOGY (O37) CLASS 12

Chapter I – Variations in Psychological Attributes (13 Marks)

Q1. What are individual differences? (2 Marks)

Individual differences refer to distinctiveness and variations among people's characteristics and behavior patterns.

Q2. What do you understand by the term situationism? (2 Marks)

Situationism is a view which states that situations and circumstances in which one is placed to influence one's behavior. For example: A person who is generally aggressive may behave in a submissive manner in the presence of his/her top boss.

Q3. What is assessment? Discuss its subtypes.

Assessment is the measurement of psychological attributes of individuals and their evaluation, often using multiple methods as standards of comparison.

Purpose

1. Predict future behavior intervention to affect a change in behavior.
2. First step in understanding a psychological attribute.

The assessment may be **Formal** or **Informal**:

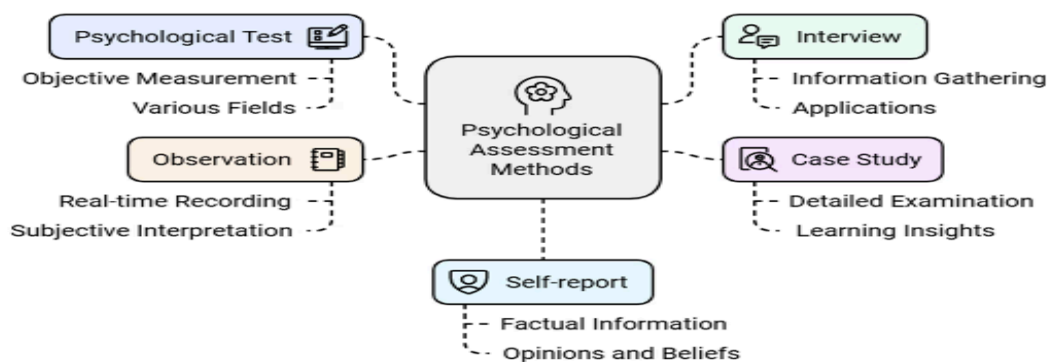
Formal assessment	Informal assessment
Objective (free from misinterpretation because of scorer's attitude, mood) Standardized (reliable, and valid), and organized. Psychologists are trained in making formal assessment.	—Open to subjective interpretation (reflects scorer's perspective) : It (interpretation) varies from case to case/one assessor to another.

Q4 Describe different domains of psychological attributes

Psychological Attributes	Definition	Uses of Test
1. Intelligence	Global capacity to understand the world, think rationally, and use available resources effectively when faced with challenges.	Provides a global measure of general cognitive competence including the ability to profit from schooling.
2. Aptitude	An individual's underlying potential for acquiring skills.	Used to predict an individual's capability if given the proper environment and training.

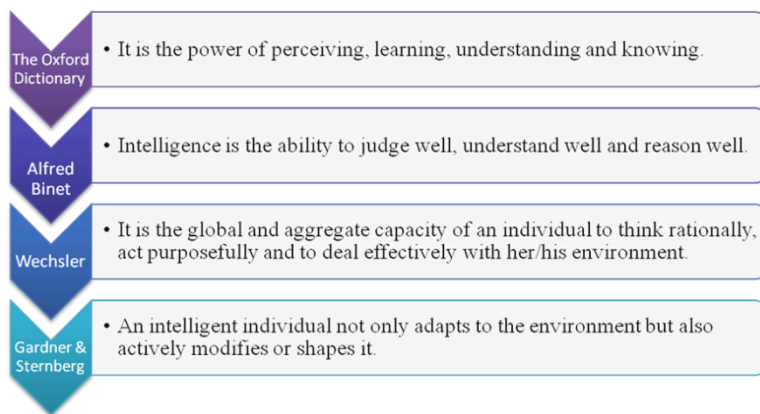
3. Interest	An individual's preference for engaging in one or more specific activities relative to others.	Helps decide what subjects/courses can be pursued comfortably and with pleasure.
4. Personality	Relatively enduring characteristics of a person that make her or him distinct from others.	Measures unique characteristics, explains and predicts future behaviour.
5. Values	Enduring beliefs about an ideal mode of behaviour.	Determines the dominant values of a person.

Q5. What are the different assessment methods used for assessment of personality



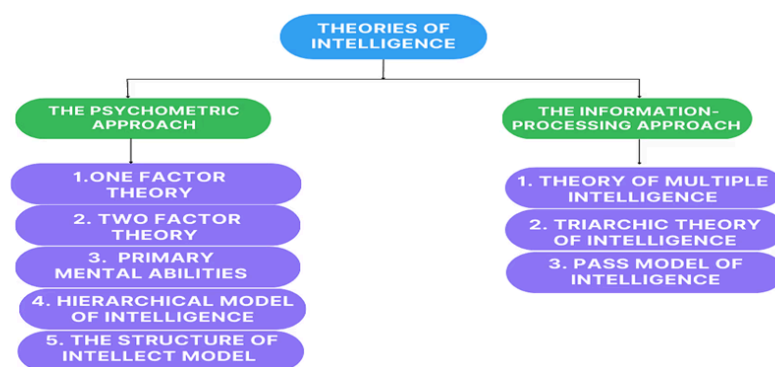
Assessment Methods	Definition	Uses of Method
1. Psychological Test	Objective and standardized measure of an individual's mental and/or behavioural characteristics.	Clinical diagnosis, guidance, personnel selection, placement, and training.
2. Interview	Seeking information from a person on a one-to-one basis.	Counsellor-client interaction, employee selection, journalism.
3. Case Study	In-depth study of the individual in terms of his/her psychological attributes, physiological history in the context of his/her psychosocial and physical environment.	Clinical psychologists, based on data generated using different methods.
4. Observation	Employing systematic, organized, and objective procedures to record behavioural phenomena occurring naturally in real time.	Disadvantages 1. Observer has little control over the situation. 2. Reports may suffer from subjective interpretations of the observer.
5. Self-Report	Person provides factual information about himself/herself, opinions, beliefs.	Obtained by an interview schedule, questionnaire, psychological test, personal diary.

Q what is intelligence ?



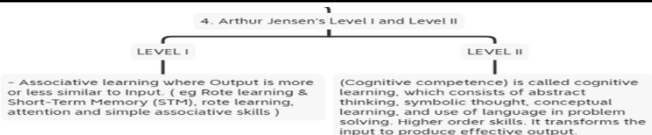
Q Differentiate between psychometric and information processing approaches of intelligence

Psychometric approach	Information processing approach
It focuses on the structure of intelligence and also known as structural approach.	It describes the processes people use in intellectual reasoning and problem solving.
It considers intelligence as an aggregate of abilities.	It focuses on how an intelligent person acts.
It expresses the individual's performance in terms of a single index of cognitive abilities.	It emphasises studying cognitive functions underlying intelligent behaviour.
Theories: Uni factor, Two factor, Theory of primary mental abilities etc.	Theories: Triarchic theory of intelligence, PASS model, etc.



Theories under psychometric approach

Theory	Founder	Description
1. Uni / one factor theory	Alfred Binet	• Definition: The ability to judge well, understand well, and reason well. • First psychologist who formalised the concept of intelligence in terms of mental operations. • Described intelligence as consisting of one similar set of abilities which can be used for solving

		any or every problem in an individual's environment.
2. Two factor theory	Charles Spearman: 1927,	As per him Intelligence consists of General factor (g- factor) are primary and common to all performances. Specific factors (s- factors) which are responsible for specific abilities- Singer, architects, scientists etc.
3. Theory of primary mental abilities	Louis Thurstone	Intelligence consists of 7 primary abilities, relatively independent. They are: 1. Verbal Comprehension (meaning, words, concepts, ideas) 2. Numerical Ability (speed & accuracy in numerical & computation skills) 3. Spatial Relations (Visualising patterns and forms) 4. Perceptual Speed (Speed in Perceiving details) 5. Word Fluency (Using words fluently n flexibly) 6. Memory (accuracy in recalling information) 7. Inductive reasoning (Deriving rules from presented facts)
4. Hierarchical model of intelligence	Arthur Jenson	 4. Arthur Jensen's Level I and Level II LEVEL I - Associative learning where Output is more or less similar to Input. (eg Rote learning & Short-Term Memory (STM), rote learning, attention and simple associative skills) LEVEL II (Cognitive competence) is called cognitive learning, which consists of abstract thinking, symbolic thought, conceptual learning, and use of language in problem solving. Higher order skills. It transforms the input to produce effective output.
5. Structure of intellect model	J.P. Guilford	According to him there are 3 dimensions of intelligence : His theory has $6 \times 5 \times 6 = 180$ cells 1. Operations- are the things that respondent does. Cognition, memory, recording, retention, convergent. 2. Contents- Nature of material or information on which intellectual operations are performed. Visual, auditory, symbolic, semantic (words) , behavioural. Products- Form in which information is processed by respondent

Theory Multiple Intelligence:

Howard Gardner: Intelligence is not a single entity, rather distinct type of intelligences exist.

Gardner studied extremely talented people and arrived on eight types of intelligences:

- 1. Linguistic (production and use of language skills):** Such people are word-smart, articulate, poets & writers.

2. **Logical-Mathematical (Scientific thinking & Problem solving):** Think Logically, critically, abstract reasoning, symbols & mathematical problem solving. Scientists, mathematicians who won nobel prize.
3. **Spatial (Visual images & Patterns):** Forming, using, transforming mental images. Pilots, sailors, sculptors, painters, architects, interior decorators, surgeons.
4. **Musical (Sensitivity to rhythm & sound pattern):** Produce, create and manipulate music patterns.
5. **Bodily-Kinaesthetic (using body flexibly & creatively):** Athletes, dancers, sportsmen, gymnasts , surgeons etc.
6. **Interpersonal (awareness of one's own feelings, motives & desires):** Using their knowledge about their strengths, limitations and using this awareness effectively to relate to others. Philosophers, spiritual leaders.
7. **Naturalists: (sensitivity towards the natural world):** Awareness of relationship with the natural world. Beauty of flora, fauna, ecology. Hunters, farmers, tourists, botanists, environmentalists, animal activists

Information processing theory/ approach

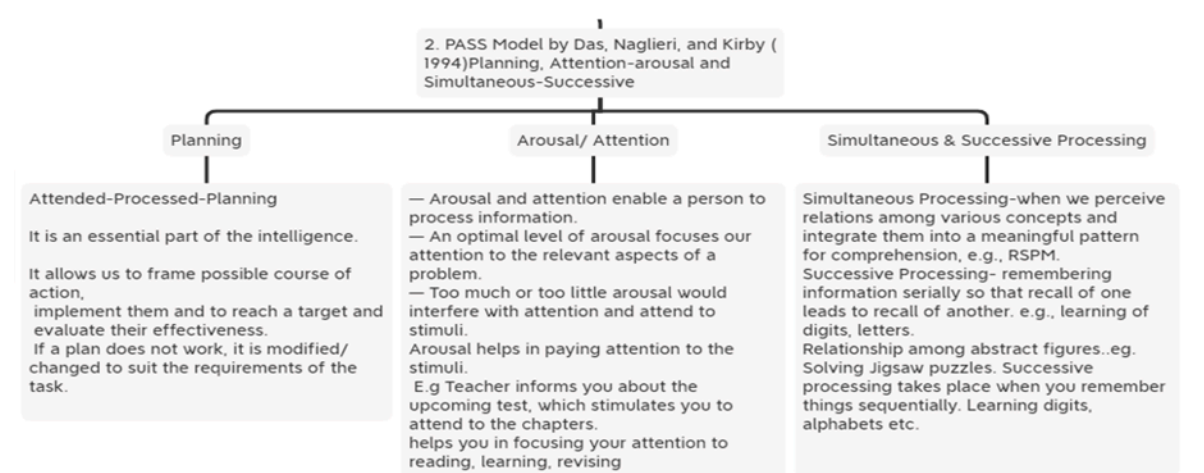
Componential / Analytical Intelligence	Experiential / Creative Intelligence	Contextual / Practical Intelligence
COMPONENTIAL SUBTHEORY Specifies the cognitive processes that underline all intelligent behaviour Analysis of information to solve problems. Persons high on this think analytically and critically and succeed in schools. This intelligence has three components: each serving different function: a) Knowledge Acquisition KNOWLEDGE ACQUISITION COMPONENTS Encode, combine and compare information (Find stage) --responsible for learning and acquisition of the ways of doing things. b) Meta components/ Higher order component METACOMPONENTS Control, monitor and evaluate cognitive processing (cognitive processing analyse & solve stage) --- planning concerning what to do and how to do it. c) Performance component PERFORMANCE COMPONENTS Execute strategies assembled by metacomponents (Operational skill- transfer stage) actually doing things	EXPERIENTIAL SUBTHEORY Specifies how experiences affect intelligence and how intelligence affects a person's experiences This uses past experiences -- Involves insights, and the ability to react to novel situations and stimuli. -- creatively to solve novel problems. — Make new discoveries and inventions. — Quickly find out what information is crucial in a given situation. For example if a person is trapped in a room, he finds out a way of coming out of the room using rope or ladder etc. in a creative way. He had some knowledge of getting out from this situation by watching out a movie few years back.	CONTEXTUAL SUBTHEORY Specifies the behaviours considered intelligent in a particular culture --- The ability to deal grasp, understand environmental demands encountered on a daily basis or everyday tasks — may be called 'street smartness' or 'business sense' — easily adapt to their present environment/select a more favourable environment, modify the environment to fit their needs. -- This is the Contextual aspect of intelligence.

Q Describe the Triarchic theory of Intelligence.

Q Any intellectual activity involves the independent functioning of three 'neurological systems'. Explain with reference to PASS model.

Robert Sternberg (1985): Intelligence is Ability to adapt, to shape and select environment to accomplish one's goals and those of society/culture.

PASS Model of intelligence: (Planning, Attention Arousal and Simultaneous- Successive model)- by J.P. Das, Jack Naglieri, Kirby



Arousal/Attention: Arousal helps in **paying attention** to the stimuli. Too much or too little arousal with interfere with the attention. **E.g Teacher informs you about the upcoming test, which stimulates you to attend to the chapters.**

Simultaneous & successive progressing: **Simultaneous** processing allows you to perceive the relationship between various concepts and integrate them into meaningful patterns. Relationship among abstract figures..eg. Solving Jigsaw puzzles.

Successive processing takes place when you remember things sequentially. Learning digits, alphabets etc.

Planning: It is an essential part of the intelligence. After the information is attended to and processed, planning is activated. It allows us to think of possible courses of action need to be implemented to reach the target and evaluate their effectiveness.

Q What is the role of nature and nurture in the development of intelligence? [4 m]

OR

Q What extent is our intelligence the result of heredity (nature) and environment (nurture)? Discuss.

Individual Differences in Intelligence:

The study conducted on Identical/fraternal twins, siblings, brought up together or in different environment helps us establish the fact that the factors that influence intelligence are:

Nature- Heredity, genes

Nurture- Environment, nutrition

Studies favouring role of Nature	Studies favouring role of Nurture
Studies done on identical twins stated that Identical twins reared together correlate almost 0.90. Twins separated early correlate 0.72, those of fraternal twins reared together correlate almost 0.60, and those of brothers and sisters reared together correlate about 0.50, siblings reared apart correlate about 0.25.	Studies done on adopted children have reported that as children grow in age, their intelligence level tends to move closer to that of their adoptive parents.
Studies done on Adopted children showed that children's intelligence is more similar to their biological rather than adoptive parents.	Studies done on environmental influences states that There is evidence that environmental deprivation lowers intelligence while rich nutrition, good family background, and quality schooling increases intelligence. Nurture

Hence, there is a general consensus amongst psychologists that Intelligence is a product of complex interaction of heredity (Nature) and environment (Nurture).

Q. What is IQ? How do psychologists classify people on the basis of IQ scores? (3/4 Marks)

Assessment of Intelligence:

1905, Alfred Binet and Theodore Simon – made the first successful attempt to measure intelligence.

MA- Mental Age- It's a measure of intellectual development relative to people of same age.

CA- Chronological Age- It's the biological age from birth.

If MA is higher than CA, the person is considered bright and more intelligent.

If MA=CA, then it is considered average intelligence.

If MA<CA, The person is called retarded as per Binet and Simon.

Intelligence Quotient: 1912, William Stern, German Psychologist: IQ

IQ= Mental Age divided by Chronological age and Multiplied by 100.

IQ= MA/CA X100

100 is the multiplier to avoid value in decimals.

So if MA=CA, IQ is 100

For value >100, it means the child's mental age is higher by those points than chronological age.

And for values <100, considered low IQ.

Variations of Intelligence:

Intellectual Deficiency

Is defined as significantly sub average general intellectual functioning with deficit in adaptive behaviour and manifested during the development period.

This impacts 3 features as captured in the definition:

1. Sub- average intellectual functionality.- Slower in grasp than children of same age.
2. Adaptability of adapt and deal with environment effectively. (Independently holding job and family)
3. Manifestation- during developmental period.

Mild- IQ 55-70 , Moderate- IQ 35-55, Severe IQ- 20-35 & Profound IQ < 20

Intellectual Giftedness

Q Differentiate between talent and giftedness. Discuss some of the characteristics of gifted children during Infancy.

Q“Roma is a 10 years old girl how has an exceptional general ability which can be seen in wide variety of areas”

a) Which ability is been discussed here? b) Explain its important characteristics in detail?

ANS Giftedness- Exceptional general ability shown in superior performance in a wide variety of areas.

Talent- is a narrower term and refers to remarkable ability in a specific field (e.g. spiritual, social, aesthetic, etc..). Highly talented people are also called Prodigies.

It is- **combination of high ability, high creativity and high commitment.**

Important characteristics of gifted children:

1. • advanced logical thinking, questioning and problem solving behaviour
2. • high speed in information processing
3. • superior generalisation and discrimination ability
4. • advanced level of original and creative thinking
5. • high level of intrinsic motivation and self esteem
6. • independent and non-conformist thinking
7. • preference for solitary academic activities for long periods

Type of Intelligence tests:

Q Differentiate between Individual test and group test

Q Differentiate between verbal test and non verbal test

Q Differentiate between culture fair test and culture biased test of intelligence.

	Individual Tests	Group Tests
1	Administered to one person at a time	Administered to several persons simultaneously
2	Administrator to establish rapport with the subject & be sensitive to his/her feelings, moods, expressions.	No opportunity to be familiar with the subject
3	In this tests subjects are allowed to answer orally/written form/manipulate objects as per tester's instructions	In this they seek written answers usually in a multiple choice .
VERBAL TESTS	NON VERBAL TESTS	PERFORMANCE TESTS
	Require use of pictures or illusions as tests items.	

● <u>R</u>equire subject to manipulate objects and other material to perform the task. ● <u>C</u>an be administered to literate as well as illiterate persons. ● <u>C</u>an be administered to literate persons only. ● <u>R</u>equire the subjects to perform some motor activity. ● <u>E</u>xample – draw a man test, Wechsler Adult Performance intelligence scale, Bhatia battery of performance etc.	● <u>C</u>an be administered to both literate as well as illiterate persons. ● <u>P</u>rovide non verbal responses in the form of pictures or illusions. ● <u>E</u>xample – Ravens Progressive Matrices, CFT, Progressive matrices etc.
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<u>CULTURE FAIR TEST</u>	<u>CULTURE- BIASED TESTS</u>
☐ These tests are appropriate for all class of people. ☐ Have very low loading of culture component. ☐ Test items respect the culture perspective of other classes. ☐ These tests do not discriminate against individual belonging to different culture.	☐ These tests are appropriate only for a particular class of people. ☐ Have a high loading of culture component. ☐ Test items do not respect the culture perspective of other classes. ☐ These tests discriminate against individual belonging to different culture.

Q “According to various psychologists’ one must guard against the erroneous practices associated with Intelligence”.Justify

MISUSES OF INTELLIGENCE TESTS

- **POOR PERFORMANCE** - poor performance on a test may attach a stigma to children thereby adversely affecting their performance and self respect.
- **DISCRIMINATING PRACTICES** – the test may invite discriminating practices from parents’ teachers and elders in society.
- **BIASNESS** – biasness of the test may underestimate the IQ of children.
- **CREATIVITY IS NOT CAPTURED** – intelligence tests do not capture creative potentialities and practical side of intelligence.
- **NOT RELIABLE** – intelligence tests are not reliable. They are not exact measures of intelligence.
- **CULTURE BOUND** – these tests are inevitably culture bound and favors the children of educated parents whereas on the other hand children whose environment provide little exposure to books , magazines and cultural interest are penalized.
- **SHORTAGE** – there is shortage of standardized intelligence tests.
- **SPEED TESTS** - Intelligence test put premium on speed. Children who are original thinkers and who can give many original alternatives answers are penalized.

Q How is the western concept of intelligence different from Indian concept of intelligence.

OR

Differentiate between ‘Technological Intelligence’ and ‘Integral Intelligence.’ Elaborate the concept of Intelligence in Indian Tradition. Are there cultural differences in the conceptualization?

Intelligence in Indian Tradition:

Unlike the western view, which particularly focuses on cognitive parameters, following competencies are identified as facets of intelligence in the Indian tradition:

- **Cognitive capacity:** (Sensitivity to context, understanding, discrimination, problem solving, effective communication)
- **Social competence:** (respect of social order, commitment to elders, the young and the needy , concern about others perspectives)
- **Emotional competence:** (Self regulation, self monitoring of emotions, honesty, politeness, good conduct and self-evaluation)
- **Entrepreneurial competence:** (commitment, persistence, patience, hard work, vigilance, goal-directed)

<u>TECHNOLOGICALLY ADVANCED CULTURE</u> (Western culture)	<u>ASIAN AND AFRICAN CULTURE</u> (Non – Western Culture)
❑ These societies promote a type of behavior called technological intelligence ❑ Involve skills like attention, observation, problem solving , decision making etc ❑ Focuses on cognitive process. ❑ Emphasize on personal or individual performance and child rearing practices.	❑ These societies promote a type of behavior called integral intelligence. ❑ Involve skills related to self action and actions related to society and others, collectivist orientation. ❑ Focuses on cognitive as well as non cognitive process ❑ Emphasize on the concept of budhi or self based on conscience, will and desires.

Q What role does Emotional intelligence play in an individual’s life?

Q “Sharad is a class XIIth boy who is generally known as the emotionally intelligent guy of the class”

a) What characteristics do you think makes Sharad an emotionally intelligent boy?

Emotional Intelligence: The notion of emotional intelligence broadens the concept of intelligence as it considers that Intelligence includes **emotions**. It is represented as EQ.

Salovey & Mayer first defined EQ as- The **ability to monitor one’s own and other’s emotions** and discriminate them to use them **to guide one’s thinking and actions**.

Characteristics of the people with high EQ:

- **Perceive and be sensitive to your feelings and emotions.**
- Sensitive to **observing others’ emotions by** noting their **body language, voice, tone & facial expressions**.
- **Use emotions** and thoughts while **solving problems and taking decisions**.
- Understand the **nature and intensity of emotions** and their powerful influence.
- **Control on emotions/feelings** while dealing with others **to achieve peace and harmony**.

Q Differentiate between Aptitude & Interest:

Aptitude is the potential to perform any activity/task

Interest is the preference for performing certain activity.

To excel in any field a person needs to have both Aptitude and Interest.

Measuring Aptitude:

There are several types of Aptitude tests largely available in two forms:

1. **Independent (specialised) aptitude tests.** – Mechanical, clerical, numerical, tests for specialized fields.
2. **Multiple (generalized) aptitude tests.**– Exist in the form of test batteries. Differential Aptitude test (DAT), General aptitude test battery (GATB), Armed services vocational aptitude battery (ASVAB) are well known.

DAT is commonly used in educational settings, consisting eight specialized subtests- **Verbal, Numerical, Abstract, Clerical speed and accuracy, Mechanical, Space, Spellings and Language tests.** (common test)

QHow is creativity related to intelligence?

OR

How creativity and creativity tests are related but different from each other

(i)**Creativity** is the ability to produce ideas, objects, or problem solutions that are novel, appropriate and useful.

(ii) **Intelligence** is subset of creativity.

(iii) Researchers have found that both high and low level of creativity can be found in highly intelligent children and also children of average intelligence. The same person can be creative as well as intelligent but it is not necessary that intelligent once must be creative.

Terman in 1920 found that Person with high IQ were not necessarily creative.(He s just a faster computer/processor).

And creative ideas can come from persons who did not have very high IQ. Ability to think differently. The relationship between Intelligence and creativity is positive. All creative abilities require a minimum level of intelligence to acquire knowledge, capacity to comprehend, retain and retrieve. e.g. to express creativity in writing, one must possess adequate language skills and to express creativity in creating new laws of science, one must have to intelligence to acquire basic knowledge of the subject.

Creativity Tests		Intelligence Tests	
•	These test use open ended questions.	•	These tests use close ended questions.
•	Maximum scope to assess expression of spontaneity, originality and imagination.	•	Measure abilities which involve convergent thinking.
•	Focus on divergent thinking.	•	Focus on assessing abilities.
•	GUILFORD, KHATANA, PARAMESH.	•	BAQER MEHDI and PASSI developed creativity tests.
•	See relationships between unrelated things.	•	Person has to think right solution.

CHAPTER 2: SELF AND PERSONALITY (13 MARKS)

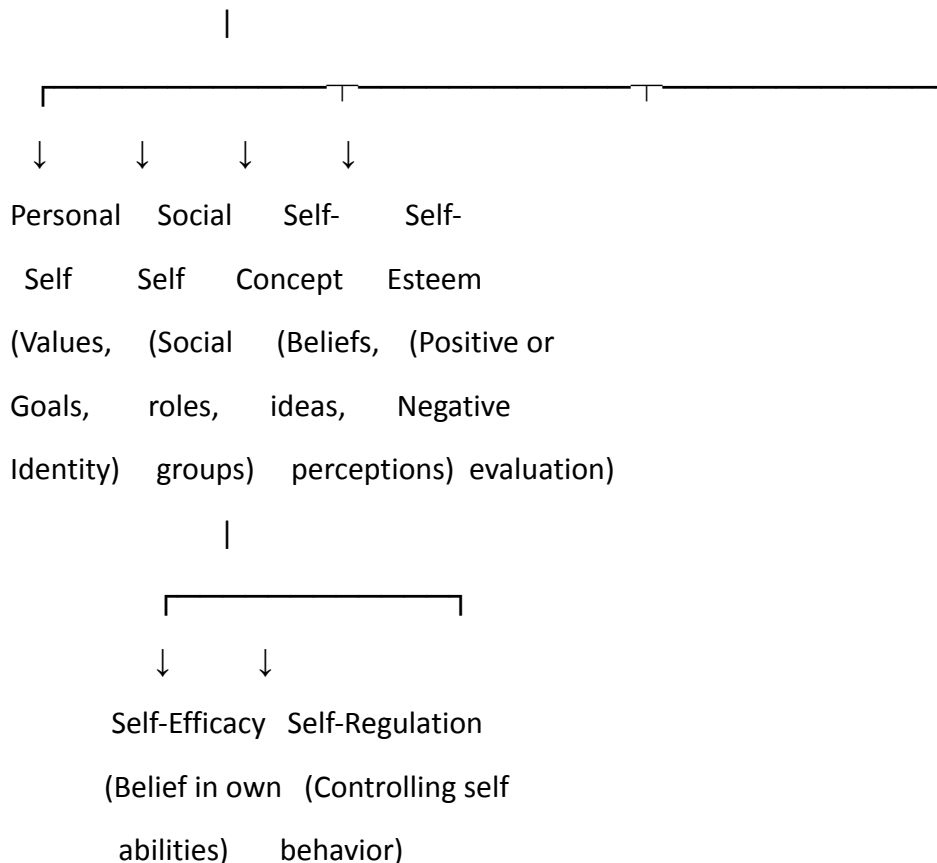
USEFUL LINKS

- <https://www.selfstudys.com/books/ncert-notes/english/class-12th/psychology/chapter-2-self-and-personality/981967>

Self refers to the totality of an individuals conscious experiences, ideas thoughts and feelings with regard to her self or him self.

- Personal identity refers to those attributes of a person that make him/her different from others.
- Social identity refers to those aspects of a person that link him/her to a social or cultural group or are derived from it.

SELF



Q What is a self?

Self refers to the totality of an individuals conscious experiences, ideas thoughts and feelings with regard to her self or him self.

Q Differentiate between self as an object and subject as a object.

Subject	Object
Who does something (actor).	Which gets affected (consequence).
Self actively engages in the process of knowing itself.	Self gets observed and comes to be known.

Q Describe different forms of self

- **Kinds of Self:**

1. **Personal Self:**

Primarily concerned with oneself.

Emphasis comes to be laid on those aspects of life that relate only to the concern the person.

2. **Social/Familial/Relational Self** Emerges in relation with others.

Emphasises such aspects of life as co-operation, unity, affiliation, sacrifice, support or sharing.

Self-concept is the way perceive ourselves and the ideas we hold about our competencies and attributes.

- **Self-esteem** is the value judgement of a person

1. By 6 to 7 years, children have formed self-esteem in four areas—academic, social and physical/athletic competence, and physical appearance become more refined with age.

- **Self-efficacy** is the extent to which a person believes they themselves control their life outcomes or the outcomes are controlled by luck or fate or other situational factors.

1. The notion of self-efficacy is based on Bandura's social learning theory. He showed that children and adults learned behaviour by observing and imitating others.

- **Self-regulation** refers to the ability to organize and monitor one's own behaviour. 1. People who are able to change their behaviour according to the demands of the environment are high on self-monitoring.

2. **Self-control** is learning to delay or refer the gratification of needs.

- **Techniques of self-control:**

1. Observation of own behaviour: provides necessary information that may be used to change, modify or strengthen certain aspects of self.

2. Self-instruction: instructs ourselves to do something and behave the way we want to.

3. Self-reinforcement: rewards behaviours that have pleasant outcomes.

Q Differentiate between Indian and Western notion of self

Indian	Western
Shifting nature of boundary between self and other (individual self and social self).	Boundary is relatively fixed.
Does not clear dichotomies.	Holds clear dichotomies between self and other, man and nature, subjective and objective.
Collectivistic culture: Self is generally not separated from one's own group;	Individualistic Culture: Self and the group exist as two different entities with clearly defined

rather both remain in a state of harmonious co-existence.	boundaries; individual members of the group maintain their individuality.
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CONCEPT OF PERSONALITY

- Personality refers to unique and relatively stable qualities that characterized an individual's behaviour across different situation over a period of time.

1. Derived from persona (Latin), the mask used by actors in Roman theatre for changing their facial make-up.

Features of Personality:

1. Personality has both physical and psychological components.
2. Its expression in terms of behaviour is fairly unique in a given individual.
3. Its main features do not easily change with time.
4. It is dynamic in the sense that some of its features may change due to internal or external situational demands; adaptive to situations.

APPROACHES TO STUDY PERSONALITY

Major approaches of Personality

1. **Type approaches** - study broad patterns in the behaviour
2. **Trait approaches** – focuses on specific attributes of individuals
3. **Interactional approach** – Situational characteristics play an important role in determining behaviour.
4. **Psychodynamic approach** – Focused on unconscious determinants of behaviour.
5. **Behavioural approach** – Personality can be understood as the response of an individual to the environment.
6. **Cultural approach** – Understand personality in relation to ecological and cultural environment.
7. **Humanistic approach** – Personality is driven by needs to adapt, learn, grow and excel.

Q Briefly discuss the type approach to personality?

• TYPE APPROACHES

1. **Hippocrates (Greek Physician)**

- (i) Proposed a typology of personality based on fluid or humour.

- (ii) Classified people into four types (i.e., sanguine, phlegmatic, melancholic and choleric); characterised by specific behavioural features.

2. Charak Samhita (Treatise on Ayurveda)

- (i) Classifies people into the categories of vata, pitta and kapha on the basis of three humoral elements called tridosha.
- (ii) Each refers to a type of temperament, called prakriti (basic nature) of a person.

3. Typology of personality based on the trigunas, i.e. , sattva, rajas, and tamas. — Sattva guna—cleanliness, truthfulness, dutifulness, detachment, discipline. — Rajas guna—intensive activity, desire for sense gratification, dissatisfaction, envy, materialism.

— Tamas guna—anger, arrogance, depression, laziness, helplessness

All the three gunas are present in every person in different degrees—the dominance of . any guna leads to a particular type of behaviour.

4. Sheldon

Using body built and temperament as the main basis for classification:

- (i) Endomorphic (fat, soft and round)—relaxed and sociable.
- (ii) Mesomorphic (strong musculature, rectangular, strong body build)—energetic and courageous.
- (iii) Ectomorphic (thin, long, fragile)—brainy, artistic and introverted.

Sheldon's Somatotype	Character	Shape	Sample Picture
Endomorph [viscerotonic]	relaxed, sociable, tolerant, comfort-loving, peaceful	plump, buxom, developed visceral structure	
Mesomorph [somatotonic]	active, assertive, vigorous, combative	muscular	
Ectomorph [cerebrotonic]	quiet, fragile, restrained, non-assertive, sensitive	lean, delicate, poor muscles	

5. Jung

Grouped people into two types, widely recognized.

- (i) **Introverts:** People who prefer to be alone, tend to avoid others, withdraw themselves in the face of emotional conflicts, and are shy.
- (ii) **Extraverts:** Sociable, outgoing, drawn to occupations that allow dealing directly with people, and react to stress by trying to lose themselves among people and social activity.

6. Friedman and Roesenman

Tried to identify psycho-social risk factors and discovered types.

- (i) **Type-A** (susceptible to hypertension and coronary heart disease): Highly motivated, impatience, feel short of time, be in a great hurry, and feel like being always burdened with work.
- (ii) **Type-B** The absence of Type-A traits.

Morris continued this research and identified:

- (iii) **Type-C** (prone to cancer): Co-operative, unassertive patient, suppress negative emotion, show compliance to authority.
- (iv) **Type-D** (prone to depression).

Q Briefly discuss the trait approach to personality

TRAIT APPROACHES

A trait is considered as a relatively enduring attribute or quality on which one individual differ another. They are:

Relatively Stable over Time

- Generally consistent across situations.
- Their strengths and combination vary across individuals leading to individual differences in personality.

1. Allport's Trait Theory (Gordon Allport)

Analysed words people use to describe themselves—provided a basic for understanding human personality—and categorized them into—

- A. **Cardinal Traits:** highly generalized disposition, indicates the goal around . which a person's entire life revolves, e.g., Hitler's Nazism.
- B. **Central Traits:** less pervasive in effect, but still quite generalized disposition. e.g, sincere.
- C. **Secondary traits-** least generalized characteristics of a person, e.g., likes mangoes.



2. Personality Factors (Raymond Cattell)

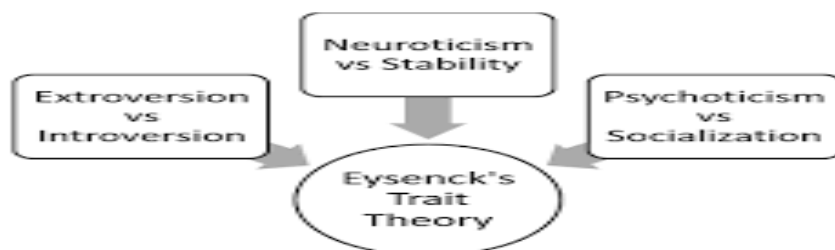
Identified two types of traits

— **Source or Primary Traits** (16): stable, building blocks of personality— described in terms of opposing tendencies.

— **Surface Traits**: result out of the interaction of source traits.

(i) Developed Sixteen Personality Factor (16PF) Questionnaire for the assessment of personality.

2. Eysenck's Theory (H.J. Eysenck)



Reduced personality into, two broad dimensions

— **Neuroticism** (anxious, moody, touchy, restless) vs. Emotional stability (calm, even tempered, reliable)—the degree to which people have control over their feelings.

— **Extraversion** (active, gregarious, impulsive, thrill seeking) vs. Introversion (passive, quiet, caution, reserved)—the degree to which people are socially outgoing or socially withdrawn.

(i) Later proposed a third dimension, **Psychoticism** (hostile, electric, and antisocial) vs. Sociability, considered to interact with the other two dimensions.

(ii) Developed Eysenck Personality Questionnaires to study dimensions of personality.

Five-Factor Model of Personality (Paul Costa and Robert McCrae)

Personality Trait	High	Low
Openness to Experience	Imaginative, curious, open to new ideas, interested in cultural pursuits.	Rigid.
Extraversion	Socially active, assertive, outgoing, talkative, fun-loving.	Shy.
Agreeableness	Helpful, co-operative, friendly, caring, nurturing.	Hostile, Self-centered
Neuroticism	Emotionally unstable, anxious, worried, fearful, distressed, irritable, hypertensive.	Well adjusted.
Conscientiousness	Achievement-oriented, dependable, responsible, prudent, hardworking, self-controlled.	Impulsive.

- **Psycho-dynamic Approach (Sigmund Freud) Important**

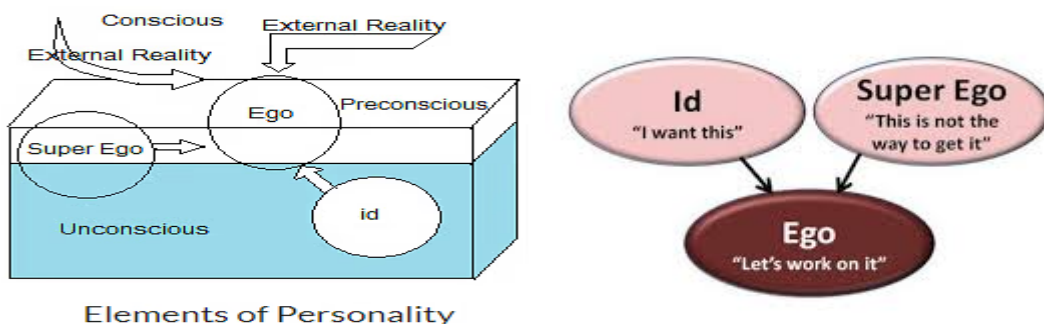
A Levels of Consciousness

1. **Conscious**—thoughts, feelings and action of which **people are aware**. **Example** , you may be conscious of the feel of your pen, sight of the pages etc.
2. **Preconscious**—mental activity which people may become aware **only if they attend to it** closely. **Example**, what you had in your breakfast today? Even though you were not aware of the breakfast before the question was asked , but it is now in your conscious.
3. **Unconscious**—mental activity that people **are not aware of**

example- Freud contended shameless experience or unacceptable sexual urges are often driven deep within unconscious.

B Structure of Personality

Freud divide the structure of personality into three parts- id,ego and superego



- **Id(WHAT WE DESIRE)**

1. Source of a person's instinctual energy—**deals with immediate gratification of primitive needs**, sexual desires and aggressive impulses.
2. **Works on the pleasure principle**, which assumes that people seek pleasure and try to avoid pain.
3. **Energised by instinctual forces**, life (sexual) instinct (libido) and death instinct.
4. **Example**-a boy who wants an ice cream will grab the cone and eat it.

- **Ego(WHAT CAN BE REALISTICALLY OBTAINED)**

1. Seeks to satisfy an individual's instinctual needs in accordance with reality.
2. **Works on the reality principle**, and directs the id towards more appropriate ways of behaving.
3. **Example** – a boy who wants an ice creame cone, will ask for permission to eat the cone.

- **Superego (WHAT OUR M,ORAL CODE TELLS US :RIGHT OR WRONG)**

1. **Moral branch** of mental functioning.

2. Tells the id and ego whether gratification in a particular instance is ethical

3. Controls the id by internalising the parental authority the process of socialisation.

4. **Example-** A boy who wants an ice cream cone, his superego will indicate whether that behaviour is morally correct. Obtaining the ice cream cone will create guilt, fear or anxiety in the boy.

C Ego Defence Mechanisms

1. A defence mechanism is a way of reducing anxiety by distorting reality unconsciously.

2. It defends the ego against the awareness of the instinctual reality.

(i) Repression:

- Anxiety provoking behaviours or thoughts are totally dismissed by the unconscious.
- It is a process of unconscious forgetfulness of our unpleasant and conflict producing behaviour or thoughts.
- Example-

1) On getting low marks, pain is dumped.

(ii) Projection:

- People attribute their own traits to others.
- Projection means to project one's thoughts, feelings, thoughts and hopes to others.
- Example

A person with strong aggressive tendencies may see other people as acting in an aggressive way towards her/him

(iii) Denial:

- A person totally refuses to accept reality.
- Examples- A person suffering from AIDS will deny his /her illness

(iv) Reaction Formation:

- A person defends against anxiety by adopting behaviours opposite to his/her true feelings.
- Also known as reversal formation
- Examples

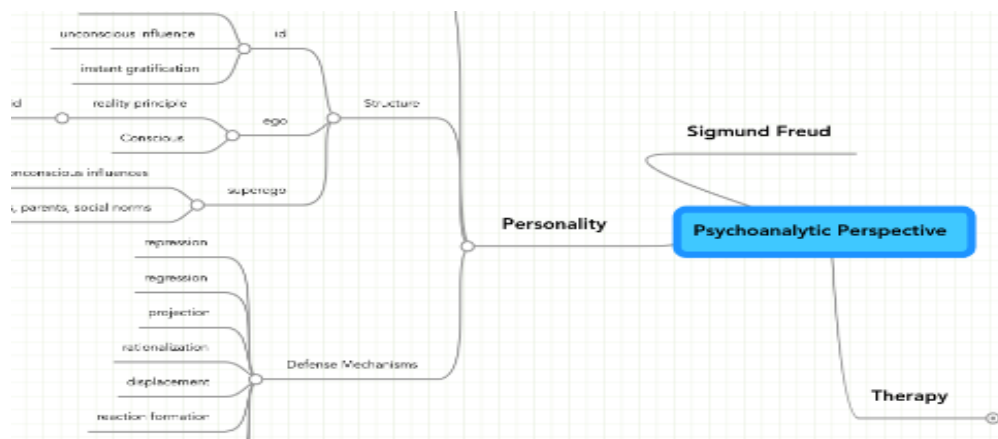
A person with strong sexual urges will channelize his/her energy into religious favour.

(v) Rationalisation:

- A person tries to make unreasonable feelings or behaviour seem reasonable and acceptable.
- Examples -A student who failed in an examination may say only crammers pass such examination.

D Stages of Personality/Psychosexual Development (Five Stage Theory of Personality)

Stage	Age	Pleasure-seeking Centre	Development
Oral	Infancy	Mouth (feeding, thumb sucking)	Basic feeling about the world are established.
Anal	2-3 years	Anus (experience pleasure in moving their bowels)	(i) Learns to respond to demands of society. (ii) Basis for conflict between the id (desire for babyish pleasure) and the ego (demand for adult, controlled behaviour).
Phallic	4-5 years	Phallus	(i) Begin to realize the differences between males and females. (ii) Become aware of sexuality and the sexual relationship between their parents.
Latency	7-Puberty		(i) Grows physically, but sexual urges are relatively inactive. (ii) Energy is channelled into social or achievement related activities.
Genital	Puberty	Genitals	(i) Attains maturity in psycho-sexual development. (ii) Sexuality, fear and repressed feeling of earlier stages are once again exhibited. (iii) Learn to deal with members of the opposite sex in a socially and sexually mature way.



• Post-Freudian Approach Neo-analytic or Post-Freudian View

- (i) Less prominent role to sexual and aggressive tendencies of the Id.
- (ii) Emphasis on human qualities of creativity, competence, and problem-solving.

1. Carl Jung: Aims and Aspirations are the source of energy

- (i) Saw human being as guided by aims and aspirations.
- (ii) Believe that personality consists of competing forces and structures within the individual rather than between the individual and the demand of society, or between the individual and reality.
- (iii) Collective unconscious consisting of archetypes or primordial images; not individually acquired, but are inherited—found in myths, dreams and arts of all mankind.

2. Karen Horney: Optimism

- Optimistic view of human life with emphasis on human growth and self actualisation
- Challenge to Freud's treatment of women as inferior—each sex has attributes to be admire by the other, and neither sex can be viewed as superior or inferior;
- Psychological disorders were caused by disturbed interpersonal relationship during childhood.
- Basic anxiety results from the environment provided by the parents.

3. Alfred Adler: Lifestyle and Social Interest source of energy-

- Individual Psychology: human behaviour is purposeful and goal directed.

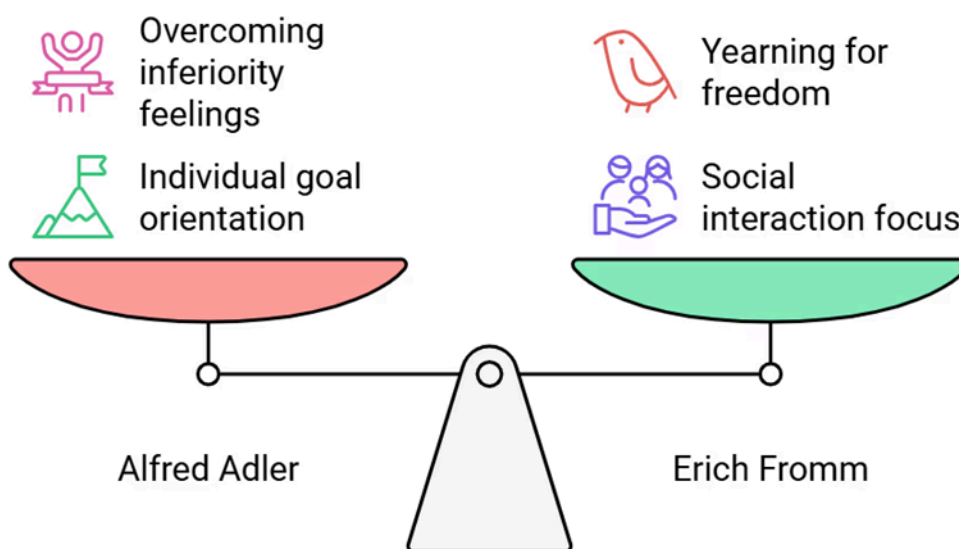
- Personal goals, goals that provide us with security and help us in overcoming the feelings of inadequacy, are the sources of our motivation.

4. Erich Fromm: The Human Concerns

- Social orientation viewed human beings as social beings who could be understood in terms of their relationship with others.
- Character traits (personality) develop from our experiences with their individuals.
- People's dominant character traits in a given work as forces in shaping the social processes and the culture itself

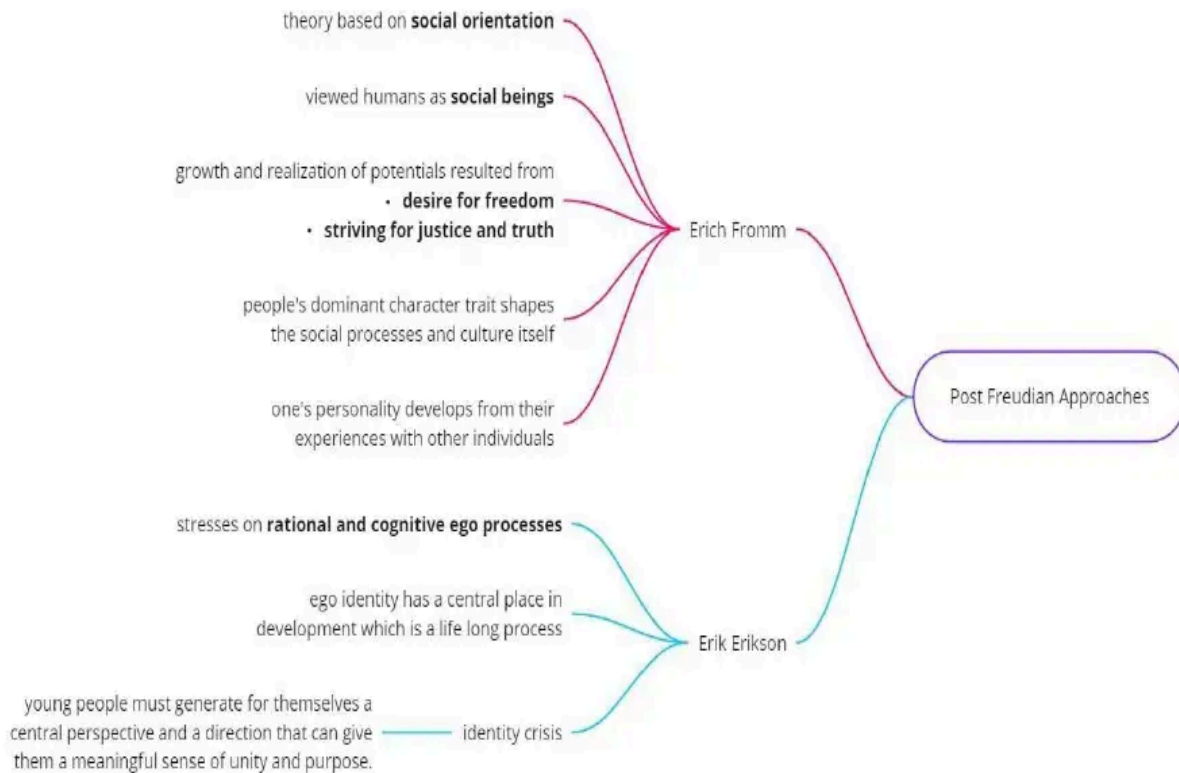
5. Erik Erikson: Search for Identity

- Rational, conscious ego processes in personality development
- Identity crisis at the adolescent age—young people must generate for themselves a central perspective and a direction that can give them a meaningful sense of unity and purpose.



Comparing Adler's individualism with Fromm's social focus.





Criticism to Psychodynamic Theories

- The theories are largely based on case studies; they lack a rigorous scientific basis.
- They use small and a typical individual as samples for advancing generalisations.
- The concepts are not properly defined, and it is difficult to submit them to scientific testing.

• Behavioural Approach

1. Focus on learning of stimulus—response connection and their reinforcement.
2. Personality is the response of an individual as sample for advancing generalization.

• Cultural Approach

- (i) Considers personality as an adaptation of individuals or group to the demand of their ecology and culture.
- (ii) A group's economic maintenance system plays a vital role in the origin of cultural and behavioural variations.

• Humanistic Approach Carl Rogers

1. Fully functioning individual—fulfilment is the motivating force for personality development (people try to express their capabilities, potentials and talents to the fullest extent possible).

Assumptions about human behaviour:

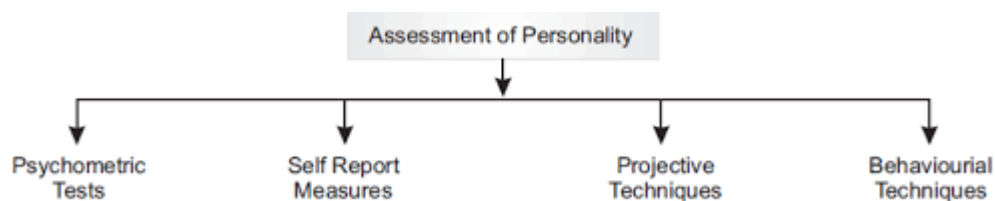
- (i) It is goal-oriented and worthwhile.
- (ii) People (who are innately good) will almost always choose adaptive, self-actualising behaviour.

• Characteristics of Healthy Person

1. Healthy become aware of themselves, their feelings, and their limits; accept themselves, and what they make of their own responsibility; have 'the courage to be'.
2. They experience the 'here-and-now'; are not trapped.
3. They do not live in the past or dwell in the future through anxious expectation and distorted defences.

• Assessment of Personality

Assessment refers to the procedures used to evaluate or differentiate people on the basis of certain characteristics.



a. Self-Report Measures:

- It was Allport who suggested that the best method to assess a person is by asking her/him about herself himself.
- The method requires the subject to objectively report her/his own feeling with respect to various items. Responses are accepted at face value, scored in quantitative terms and interpreted on basis of norms for the test.

• Uses of Self-report test:

1. Career guidance, vocational exploration and occupational testing for students/adults.

Limitations of Self-report tests:

1. Social desirability: this is a tendency on part of a student to endorse/select responses basis socially desirable behaviour.
2. Acquiescence: It is a tendency of the subject of saying Yes to items irrespective of the content, which makes it less reliable for an effective outcome.

MINNESOTA MULTIPHASIC PERSONALITY INVENTORY

—> Developed **by HATHAWAY and McKINLEY**

—> Effective in identifying varieties of psychopathology

—> Revised version is MMPI-2

—> Consists of 567 statements. The subject has to judge each statement as 'true' or 'false'. —> The test is divided into 10 sub scales which seek to diagnose hypochondriasis, depression, hysteria, psychopathic deviant, masculinity-femininity, paranoia, psychasthenia, schizophrenia, mania and social introversion.

—> **In India, Mallick and Joshi have developed Jodhpur Multiphasic Personality Inventory. (JMPI)**

EYSENCK PERSONALITY QUESTIONNAIRE

—> Developed by **Eysenck**

—> Initially assessed 2 dimensions of personality: introversion-Extraversion and emotionally stable-emotionally unstable. Emotional stability instability.

—> These dimensions are characterised by **32 personality** traits.

—> Later on, Eysenck added a third dimension, called psychoticism. It is linked to psychopathology-sociability.

—> It represents a lack of feeling for others, a tough manner of interacting with people, and a tendency to defy social conventions. A person scoring high on this dimension tends to be hostile, egocentric and antisocial.

SIXTEEN PERSONALITY FACTOR QUESTIONNAIRE(16PF)

Developed by Catell.

Used factor analysis to identify the basic personality structure.

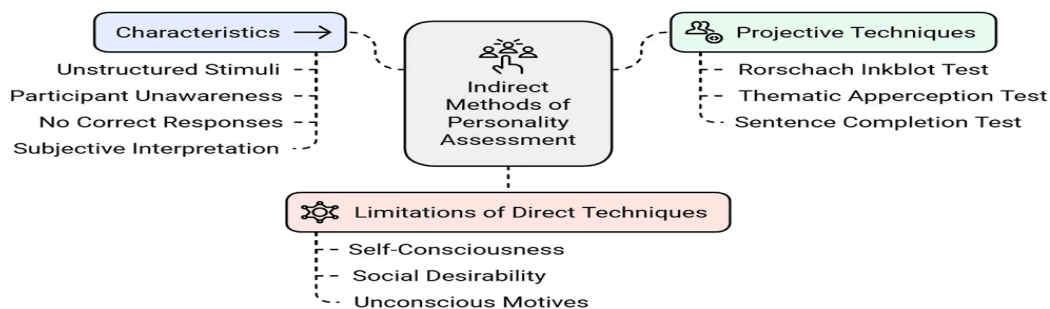
It provides declarative statements where a subject responds to specific situations by choosing from a set of given alternatives.

Can be used with high school students as well as adults.

Helpful in career guidance, vocational exploration and occupational testing

PROJECTIVE TECHNIQUE:

This technique is an **indirect method**, used to uncover and assess the large part of the behaviour which is governed by unconscious motives, as direct (self-report) methods cannot assess this.



Methods include: Reporting association with stimuli- words, inkblots, story writing around pictures, some require sentence completion, expression through drawings.

Features of this technique:

1. The stimuli are relatively or fully unstructured and poorly defined.
2. The subject is not told about the purpose of assessment and method of scoring and interpretation.
3. The person is informed that there is no correct or incorrect answer.
4. Each response is considered to reveal a significant aspect of personality.
5. Scoring and interpretation are lengthy and sometime subjective.

Examples of Projective tests:

1. **Rorschach Inkblot Test:**

This test was developed by **Hermann Rorschach**. The tests consists of 10 inkblots (5 black and white, 2 red and remaining of pastel colours) printed in the centre of a cardboard of 7" to 10". **1st**

Phase- Performance proper: Subjects are shown the cards and are asked to tell what they see in each.

2nd Phase- Inquiry: A detailed report of responses is prepared by asking the subject to tell on where, how and on what basis was a particular response made.

2.The Thematic Appreciation Test (TAT): developed by **Morgan and Murray**. Little more structured than the Inkblot test. It consists of **30 black and white picture** cards and 1 blank card. Each card depicts one or more people in a variety of situations. 20 cards to 5 cards are used for performing assessment. Method: One card is presented at a time, asking the subject to tell a story describing the situation presented in the picture:

What led up to the situation

What is happening at the moment

What will happen in future

A standard procedure is followed for scoring the TAT responses.

Indian adaptation done by: Uma Chaudhary.

3. Rozensweig's Picture-Frustration study (P-F Study): was developed by **Rozenweig** to assess how people express aggression in a frustrating situation.

The test consists cartoon like pictures depicting situations where one person is frustrating other.

The subject is asked to describe:

What the frustrated person will say or do?

Analysis is based on:

1. the Type and Direction of aggression (towards oneself or environment or evading the situation).
2. It is examined whether the focus is on frustrating object or protecting the frustrated person, or on constructive solution.

4. Sentence Completion Test:

This test makes use of number of incomplete sentences. The starting of the sentence is presented and the subject has to provide an ending of the sentence. The type of ending helps assess the unconscious attitude, motivation and conflicts.

e.g.

1. My father.....
2. My greatest fear is.....

5.Draw-a-Person test:

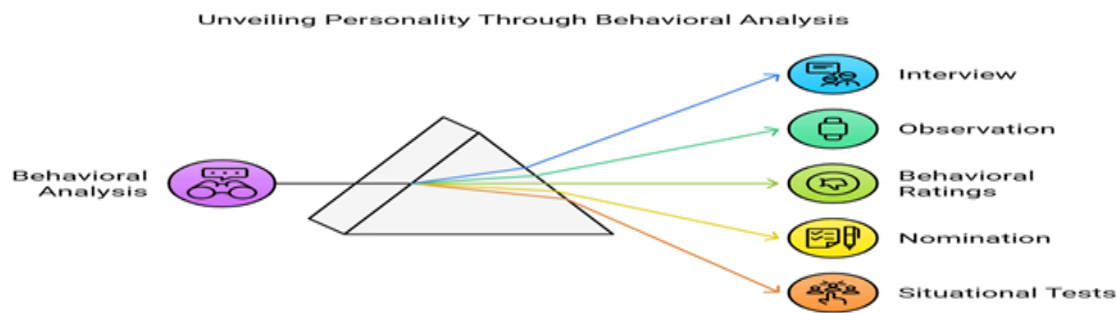
In this test subject is provided with a pencil, eraser and sheet and asked to draw a picture of a person.

After the completion of the drawing, subject is asked to draw a picture of a person of opposite gender. Subject is asked to make a story about the person as if he/she was a character of a movie/novel. Some examples of the interpretation as follows:

1. Omission of facial features suggests that the person tries to evade a highly conflict-ridden interpersonal relationship.
2. Graphic emphasis on the neck suggests lack of control over impulses.
3. Disproportionately large size of the head suggests organic brain disease or preoccupation with headaches.

Behavioural Analysis:

This analysis can provide us with a meaningful information about his/her personality.



An observer's report contains data obtained from:

1. **Interview:**

Structured interview follows a set of very specific questions and set procedure. This is often done to make objective comparison of persons being interviewed. Use of rating scales add to the objectivity.

Unstructured Interview involves asking a number of questions (not specific) to develop an impression about a person. The way a subject answers and presents himself and answers the questions carries enough potential to reveal about his/her personality.

2. **Observation:**

Use of **Observation for a personality assessment** is a sophisticated procedure that cannot be carried out by untrained people. It requires careful training of the observer and fairly detailed guideline to carry out analysis to use observations to assess personality. In spite of the widespread use of this method, it has following **limitations**:

1. Professional training required for collection of useful data and is quite demanding and time consuming.
2. Maturity of the observer is a precondition. Else personal biases can alter the assessment.
3. Mere presence of the observer may contaminate the results.

3. **Behavioural Ratings**

Behavioural ratings are frequently used for personality assessment of individuals in an educational or industrial settings.

Behavioural ratings are generally taken from the people who know the assessee intimately and have interacted over a period of time. In order to use ratings the traits should be clearly defined in terms of carefully stated behavioural anchors.

Limitations of Behavioural Rating method:

1. Raters generally display biases that colour their judgements of different traits. For example most of are greatly influenced by a single favourable/unfavourable trait which colours the overall judgment on all the traits. This is called 'Halo effect.'
2. Raters have a tendency to place individuals in the middle of the scale (middle category bias) or in the extreme positions (called extreme response bias).

4. Nominations: in this method people in a group who know each other for a long period are asked to nominate another person from the group with whom they would like to work/play/do some activity. Then they are asked to state the reason why they would have nominated that person.

5. Situational tests: A variety of situational tests have been devised for the assessment of personality. In performing this test the person is given a task under stressful environment, where others are instructed not to provide any support and act non-cooperative. This is kind of role playing. The subject is observed and a report is prepared. Situations can be videotaped and observed for assessment later.

Chapter 3 – Meeting Life Challenges (9MARKS)

Q Explain the concept of stress.

Stress definition: The pattern of responses our organisms make against the stimulus/event that disturbs the equilibrium and exceeds a person's ability to cope. **Nature of Stress:** The word stress originated from Latin word- 'strictus' meaning tight, narrow – internal feeling of tightness, constriction of muscles and breathing reported by many people under stress.

Stressor: is an event that causes our body to give the stress response. E.g.

- a. External- environment (noise, pollution, crowd etc.)
- b. Social- Break-up with a friend, loneliness

c. Psychological- Mental conflict, frustration

Strain: is the reaction of our body to external stressors.

Q. Briefly discuss the Lazarus model of stress .

Lazarus described two types of appraisals– primary and secondary:

Primary Appraisal: refers to the perception of a new or changing environment as **Positive, Neutral or Negative** in its consequences.

Secondary Appraisal: is when we perceive an event very stressful that it involves challenge appraisal that involve more confident expectations of ability to cope up. **This is more focused on coping abilities and assessment of resources**-mental, physical, social or personal to meet the challenge of harm, threat or challenge of the event.

Negative events are appraised for their possible **harm, threat or challenge**.

Harm- is the assessment of damage that has been done by the event.

Threat– Possible future damage by the event

Challenge– more confident expectations of the ability to cope up with the stressful event.

Q Explain different types of stress.

Types of Stress:

- 1. Physical stress:** Impact state of our body or strain our body e.g. we overexert, lack of nutrition/diet, suffer an injury or fail to get enough sleep.
- 2. Environmental stress:** is caused by our surroundings. E.g. pollution, crowding, noise, heat of the summer, winter cold or catastrophic events like floods, fire and earthquakes etc.
- 3. Psychological Stress: (imp)** These are generated in our minds and are personal and unique to the person. Caused by **frustration, conflicts, internal and social pressures** they lead to worry, anxiety or depression.
 - a. Frustration:** Results from blocking of needs and motives and someone or something hindering us from achieving goals. E.g. social discrimination, interpersonal hurt, low grades in school. Etc.
 - b. Conflicts:** Occur between two or more incompatible needs. E.g. to study Dance or Psychology, to continue with old or take up a new job, to do something against the personal values.
 - c. Internal Pressures:** are the stresses caused by high expectations we set from ourselves or setting unrealistic goals. E.g. trying to do everything perfectly, setting high goals.
 - d. Social Pressures:** Excessive demands from people around us in family, community. Working with people with interpersonal conflicts and personality clash.

Social Stress: These are caused by people in society and surroundings. Social events like death in the family, conflict with the neighbourhood, strained relationship between spouses.

Other questions

- 1. Briefly discuss the different types of psychological stress (4 Marks)**
- 2. Differentiate between internal pressure and social pressure. (2 Marks)**
- 3. Briefly discuss the various types of social stress (2 Marks)**

Q. State the sources of stress. (3 Marks)- Important

Sources of Stress:

A wide range of events and conditions can generate stress.

- 1. Life Events:** any event causing major and sudden disruption in our routine and life can cause stress as we find it difficult to cope with that sudden change. e.g. break up in a long-term relationship, business exigency.
- 2. Hassles:** Personal stresses we ensure as individuals due to happenings in our daily lives. E.g traffic while commuting, quarrelsome neighbour, electricity, water shortage etc.

3. Traumatic Events: Extreme events like fire, train accident, plain hijack, robbery etc. Such things haunt people in their dreams and keep coming back as flashbacks and cause stress.

Q. What are the effects of stress on psychological functioning (4 Marks)- Important
Effects of Stress on Psychological Functioning and Health

1. Emotional Effects: Mood swings, erratic behaviour, low confidence, alienation from family and friends, anxiety, physical tension.

2. Psychological Effects: Under psychological stress human body produces certain hormones in access- Adrenalin and cortisol. They produce marked changes in heart rate, metabolism, blood pressure and physical activity.

3. Cognitive Effects: If pressures due to stress continue, one may suffer from mental overload. This can impact individuals' ability to make sound decisions and lead to poor concentration, reduced short term memory capacity.

4. Behavioural Effects: Stress also affects our behaviour. A stressed individual tends to eat less nutritional food, increasing intake of stimulants like- cigarettes, caffeine, alcohol and other addictive substances. Dizziness and disrupted sleep patterns.

Stress and Health

Chronic daily Stress can affect physical and mental health of an individual.

Physical exhaustion can cause chronic fatigue, weakness and low energy.

The mental exhaustion appears in the form of irritability, anxiety, feelings of helplessness and hopelessness.

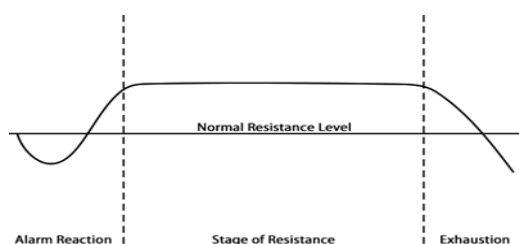
Burnout: And state of physical, emotional and psychological exhaustion is known as **burnout**. Stress can also impact our immune system and increase the chances of becoming ill, development of cardiovascular disorders, high blood pressure and sometimes psychosomatic disorders like ulcers, asthma, allergies and headaches.

Q. Describe the GAS Model and illustrate the relevance of this model with the help of an example (4 Marks) -Very important.

Selye's theory of General Adaptation Syndrome(GAS) (MOST IMPORTANT)

Selye noticed a similar pattern in bodily responses by all in their response to stress.

According to him **GAS involves three stages:**



1. Alarm Reaction:

Presence of stimulus or stressor leads to activation of adrenaline-pituitary- cortex system. This triggers the releasing of hormones to respond to stress making individual to **Fight or Flight**.

Noxious stimulus or stressor> activation of adrenaline-pituitary- cortex system> Fight or Flight mode

2. Resistance Stage:

If stress is prolonged this stage begins. **The parasympathetic system calls for more cautious use of body's resources.** The organism makes efforts to cope with the threat, through confrontation.

Prolonged Stress> Parasympathetic system uses body resources cautiously > Organism confronts the threat and attempts to cope up.

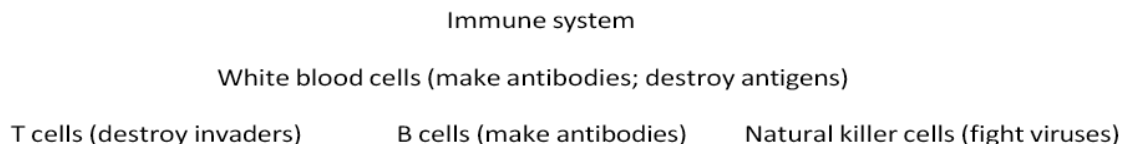
3. Exhaustion Stage:

Continuous exposure to same stressor> drains body resources> Alarm reaction & Resistance do not work> lead to stress related diseases like blood pressure etc.

Selye's model has been criticised for assigning a very limited role. They believe studying perceptions, personalities and biological factors also constitute to the response against stress.

Q. Discuss the relationship between stress and immune system .

Stress and Immune system:



The Immune system guards the body against attackers both from within and outside.

Psychoneuroimmunology focuses on the links between the mind, the brain and the immune system. It focuses on the study of how our immune system works. The white blood cells (leucocytes) within the immune system identify and destroy foreign bodies (antigens) such as viruses by producing antibodies.

- T- cells destroy invaders and enhance immunological activity. (It is these T cells when attacked by HIV virus , it causes AIDS.
- B-cells produce anti-bodies.
- Natural Killer cells are involved in fight against viruses and tumours.

Lifestyle:

Lifestyle is the overall pattern of decisions and behaviours that determine a person's health and quality of life. Stressed individuals are more likely to be exposed to pathogens – the agents causing physical illness.

Stressed people have poor nutritional habits, unhealthy sleep pattern and health risking behaviours like smoking and excessive drinking. These habits adversely impact physical and mental health. Alternately, studies reveal health promoting behaviours are like balance diet, regular exercise, family support. Adhering to this lifestyle enhances health and longevity.

Q. Enumerate different ways to cope with stress (4 Marks/ 6 Marks)

Coping With Stress:

Over the years, research shows that **it is not the Stress that we experience** that influences our health and wellbeing, **it is how we Cope with the Stress** that impacts us.

Endler and Parker suggests 3 step coping strategies:

Coping: is a dynamic situation specific reaction to stress. It is a set of concrete responses to stressful situation or events that are intended to resolve the problem and reduce stress.

The three coping strategies given by Endler and Parker are:

	Process	Example
--	---------	---------

<u>Task-oriented strategy</u>	<u>This involves obtaining information about the stressful situation and about alternative courses of action and their probable outcome.</u> It also involves deciding priorities and acting so as to deal directly with the stressful situation.	I will schedule my time better.
<u>Emotion-oriented strategy</u>	<u>This can involve efforts to maintain hope and to control one's emotions.</u>	Tell myself that it is not really happening to me, or worry about what I am going to do.

	b) It can also involve venting feelings of anger and frustration, or deciding that nothing can be done to change things.	
Avoidance-oriented strategy	) This involves denying or minimising the seriousness of the situation .) It also involves conscious suppression of stressful thoughts and their replacement by self-protective thoughts.	Watching TV, phone up a friend, etc.

Lazarus and Folkman coping strategy:

According to them coping responses can be divided into **two types**: problem focused & emotion focused.

	Definition	Example
Problem-focused	) Problem-focused strategies attack the problem itself , with behaviours designed to gain information, to alter the event, and to alter belief and commitments.) They increase the person's awareness, level of knowledge, and range of behavioural and cognitive coping options.) They can act to reduce the threat value of the event.	For example "I made a plan of action and followed it".
Emotion-focused	Emotion-focused strategies call for psychological changes	For example "I did some things to let it out of my system"

Stress Management Techniques(IMPORTANT)

Stress is a silent killer. Hypertension, heart disease, ulcers, diabetes, cancer all are linked to stress. Due to our lifestyle changes they are on the rise. In order to reduce stress we need to make some lifestyle changes:

- 1. Relaxation Technique:** Deep breathing is used along with muscle relaxation to calm the mind and relax the body.
- 2. Meditation procedures:** A yogic method of meditation consists of a sequence of techniques to bring about an altered state of consciousness. It involves thorough concentration that meditator becomes unaware of any outside stimulation and reaches in a different state of consciousness.
- 3. Biofeedback:** training involves 3 stages:
 - 1. Developing awareness** of a particular physiological response (e.g. situations when my heart rate goes up)
 - 2. Learning ways of controlling** that in quiet conditions.
 - 3. Transferring this control into actual** situations by practice.
- 4. Creative Visualization:** It is an effective technique for dealing with stress. This involves creative visualization of any new situation that is susceptible to cause stress. This makes it easier to plan and address the stressors and convert imagination into reality.

5. Behavioural technique: involves inoculate people against stress. 3 steps:

- Assessment: discussing the nature of problem and seeing it from a view point of external/third person.
- Stress reduction: involves learning the technique by approaching the solution and using relaxing technique
- Application and follow through: by applying the solution by setting self-instructions.

6.Exercise: Regular exercise improves efficiency of heart, enhances the function of lungs and maintains good circulation and metabolism. Swimming, walking, running, cycling, skipping etc help make systems stronger to combat physical impact of stress.

Promoting Positive Health and Wellbeing:

To stay healthy, we need to have Stress resistant Personality – It consists of 3 Cs: I.e. Commitment, Control and Challenge.

But not everyone has this personality. Regular people need to acquire **Life Skills** in order to manage stress in day to day life:

Life Skills: are the abilities for adaptive and positive behaviour that enable individuals to deal effectively with the demands and challenges of everyday life.

These life skills are–

	Skill	Definition	Process
Assertiveness	Assertiveness is a behaviour or skill that helps to communicate, clearly and confidently, our feelings, needs, wants, and thoughts.	It is the ability to say no to a request, to state an opinion without being self-conscious, or to express emotions such as love, anger, etc. openly.	If you are assertive, you feel confident, and have high self-esteem and a solid sense of your own identity.
Time management	The way one spends time determines the quality of one's life. Learning how to plan time and delegate can help to relieve the pressure.	The major way to reduce time stress is to change one's perception of time. The central principle of time management is to spend your time doing the things that you value, or that help you to achieve your goals.	It depends on being realistic about what you know and that you must do it within a certain time period, knowing what you want to do, and organising your life to achieve a balance between the two.
Rational thinking	Many stress-related problems occur as a result of distorted thinking.	The way one thinks and feels are closely connected. When we are stressed, we have an inbuilt selective bias to attend to negative thoughts and images from the	Some of the principles of rational thinking are: - Challenging your distorted thinking and irrational beliefs

		past, which affect our perception of the	
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Q What are the characteristics of a stress resistant personality (3 Marks)-Important

- (i) Stress Resistant Personality (Kobasa):
- (ii) Hardiness is a set of belief about oneself, the world and how they interact
People with high levels of stress but low levels of illness share three characteristics, which are referred to as the personality traits of hardiness (a set of beliefs about oneself, the world, and how they interact).
- (i) **Commitment** (personal commitment to work, family, hobbies and social life).
- (ii) **Control** (control over sense of purpose and direction in life).
- (iii) **Challenge** (changes in life as normal and positive rather than as a threat).
- (iii) Everyone does not have these characteristics, many of us have to relearn these characteristics and certain skills such as rational thinking and assertiveness to equip ourselves cope with demands of everyday life .

PSYCHOLOGICAL DISORDERS (CH 4)

**Q.1 WHAT IS ABNORMALITY & WHAT ARE THE VARIOUS APPROACHES USED TO
DISTINGUISH BETWEEN ABNORMAL & NORMAL BEHAVIOUR**

1. Definitions of abnormality have certain common features, often called the 'Four
Ds': **Deviance** : Different, extreme, unusual & even bizarre

Distress : Unpleasant & upsetting to the person and to others

Dysfunction : Interfering with the person's ability to carry out daily activities in a constructive way

Dangerous : To the person or to others

2. The word 'abnormal' literally means "**away from the normal**" & implies deviation from some clearly defined norms or standards.
3. There are **2** basic & conflicting views regarding the difference between normal and abnormal behaviours:

Deviation From Social Norms	Maladaptive Behaviours
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• Many psychologists have stated that abnormal is simply a label that is given to a behaviour which is deviant from social expectations. • Each society has norms, which are stated or unstated rules for proper conduct. A society's norms grow from its particular culture — its history, values, institutions, habits, skills, technology, and arts. • Abnormal behaviour, thoughts and emotions are those that differ markedly from a society's ideas of proper functioning. Behaviours, thoughts and emotions that break societal norms are called abnormal. Example : A society whose culture values competition and assertiveness may accept aggressive behaviour as perfectly normal while one based on community values may consider them a deviation.	• It views abnormal behaviour as maladaptive. Many psychologists believe that the best criterion for determining the normality of behaviour is whether it fosters the well-being of the individual and eventually of the group which s/he belongs. • Well-being is not simply maintenance and survival but also includes growth and fulfilment i.e. the actualisation of potential. According to this criterion conforming behaviour can be seen as abnormal if it is maladaptive i.e. if it interferes with optimal functioning and growth. Example : A student in the class prefers to remain silent even when she has questions in mind. • Describing behaviour as maladaptive implies that a problem exists. It also suggests that vulnerability in the individual, inability to cope, or exceptional stress in the environment have led to problems in life. Example : Many engage in adrenaline sports like skydiving which can be threatening to oneself but we don't classify them as abnormal.
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Q.2 WHAT IS STIGMA

1. People have vague ideas about psychological disorders that are characterised by superstition, ignorance and fear.
2. It is commonly believed that a psychological disorder is something to be ashamed of but in reality a psychological disorder which indicates a failure in adaptation should be viewed just as any other illness.
3. The stigma attached to mental illness means that people are hesitant to consult a doctor or psychologist because they are ashamed of their problems.

Q.3 TRACE THE HISTORICAL BACKGROUND OF PSYCHOLOGICAL DISORDERS?

A) SUPERNATURAL SOURCES

- 1) One ancient theory that is still encountered today holds that abnormal behaviour can be explained by the operation of supernatural and magical forces such as evil spirits (bhoot-pret), or the devil (shaitan). Exorcism, i.e. removing the evil that resides in the individual through counter magic and prayer, is still commonly used.
- 2) In many societies, the shaman, or medicine man (ojha) is a person who is believed to have contact with supernatural forces and is the medium through which spirits communicate with human beings.
- 3) Through the shaman, an afflicted person can learn which spirits are responsible for her/his problems and what needs to be done to appease them.

B) BIOLOGICAL/ORGANIC APPROACH

- 1) There has been a recurring belief that individuals behave strangely because their bodies and their brains are not working properly.
- 2) This is the biological or organic approach. In the modern era, there is evidence that body and brain processes have been linked to many types of maladaptive behaviour.
- 3) For certain types of disorders, correcting these defective biological processes results in improved functioning.

C) PSYCHOLOGICAL APPROACH

- 1) It holds that psychological problems are caused by inadequacies in the way an individual thinks, feels, or perceives the world.

D) ORGANISMIC APPROACH

- 1) Philosopher- physicians of ancient Greece such as Hippocrates, Socrates, and in particular Plato developed the organismic approach and viewed disturbed behaviour as arising out of conflicts between emotion and reason.
- 2) Galen elaborated on the role of the four humours in personal character and temperament. According to him, the material world was made up of four elements- earth, air, fire, and water which combined to form four essential body fluids- blood, black bile, yellow bile, and phlegm. Each of these fluids was seen to be responsible for a different temperament. Imbalances among the humours were believed to cause various disorders.
- 3) This is similar to the Indian notion of the three doshas of vata, pitta and kapha which were mentioned in the Atharva Veda and Ayurvedic texts.

E) MIDDLE AGES

- 1) During the early middle Ages, the Christian spirit of charity prevailed and St. Augustine wrote extensively about feelings, mental anguish and conflict.
- 2) In the middle Ages, demonology and superstition gained renewed importance in the explanation of abnormal behaviour. Demonology related to a belief that people with mental problems were evil and there are numerous instances of 'witch-hunts' during this period.
- 3) Laid the groundwork for modern psychodynamic theories

F) RENAISSANCE HUMANISM

- 1) The Renaissance Period was marked by increased humanism and curiosity about behaviour.
- 2) Johann Weyer emphasised psychological conflict and disturbed interpersonal relationships as causes of psychological disorders. He also insisted that 'witches' were mentally disturbed and required medical, not theological, treatment.

G) SCIENTIFIC APPROACH, REFORM AND DEINSTITUTIONALISATION

- 1) The seventeenth and eighteenth centuries were known as the Age of Reason and Enlightenment, as the scientific method replaced faith and dogma as ways of understanding abnormal behaviour.
- 2) The growth of a scientific attitude towards psychological disorders in the eighteenth century contributed to the Reform Movement and to increased compassion for people who suffered from these disorders. Reforms of asylums were initiated in both Europe and America.
- 3) Deinstitutionalisation in the reform movement placed emphasis on providing community care for recovered mentally ill individuals.

H)INTERACTIONAL/BIO-PSYCHO-SOCIAL APPROACH

- 1) There has been a convergence of these approaches, which has resulted in an interactional, or bio- psycho-social approach.
- 2) From this perspective, all three factors, i.e. biological, psychological and social play important roles in influencing the expression and outcome of psychological disorders.

Q.4 WHAT IS CLASSIFICATION ? HOW DO WE CLASSIFY DISORDERS? WHAT ARE THE ADVANTAGES OF CLASSIFICATION

MEANING OF CLASSIFICATION

1. Classification of psychological disorders consists of a list of categories of specific psychological disorders grouped into various classes on the basis of some shared characteristics.

CLASSIFYING DISORDERS

A) Diagnostic & Statistical Manual of Mental Disorders 5th Edition (DSM-5)

1. The American Psychiatric Association (APA) has published an official manual describing and classifying various kinds of psychological disorders.
2. The current version of it, the Diagnostic and Statistical Manual of Mental Disorders 5th Edition (DSM-5) presents discrete clinical criteria which indicate the presence/absence of disorders.

B) International Classification of Diseases (ICD-10)

1. The classification scheme officially used in India and elsewhere is the tenth revision of the International Classification of Diseases (ICD-10), which is known as the ICD-10 Classification of Behavioural and Mental Disorders.
2. It was prepared by the World Health Organisation (WHO).
3. For each disorder, a description of the main clinical features or symptoms, and of other associated features including diagnostic guidelines is provided in this scheme.

ADVANTAGES

1. Enable psychologists, psychiatrists and social workers to communicate with each other about the disorder.
2. Help in understanding the causes of psychological disorders and the processes involved in their development and maintenance.

Q.4 WHAT ARE THE CURRENT APPROACHES TO EXPLAINING ABNORMAL BEHAVIOUR

	TYPE OF MODEL	BASIS/PRINCIPLE	THEORIES
Biological Model	Biological	) A wide range of biological factors such as faulty genes, endocrine imbalances, malnutrition, injuries and other conditions may interfere with normal development and functioning of the human body.) According to this model, abnormal behaviour has a biochemical or physiological basis.	) Psychological disorders are often related to problems in the transmission of messages from one neuron to another.) A tiny space called synapse separates one neuron from the next, and for a message to move across that space, a chemical called a neurotransmitter must be released.) When an electrical impulse reaches a neuron's ending, the nerve ending is stimulated to release a chemical called a neurotransmitter.) Abnormal activity by certain neurotransmitters can lead to specific psychological disorders: - Anxiety disorders have been linked to low activity of gamma aminobutyric acid (GABA), - Schizophrenia linked to excessive activity of dopamine - Depression linked to low activity of serotonin.
Genetic Factors	Biological	) Many genes combine to help bring about our various behaviours and emotional reactions, both functional and dysfunctional.) But it appears that in most cases, no single gene is responsible for a particular behaviour or a psychological disorder.	) Genetic factors have been linked to mood disorders, schizophrenia, mental retardation and other psychological disorders.

Psychodynamic Model	Psychological (These models maintain that psychological and interpersonal factors have a significant role to play in abnormal behaviour)	) Psychodynamic theorists believe that behaviour, whether normal or abnormal, is determined by psychological forces within the person of which s/he is not consciously aware.) These internal forces are considered dynamic, i.e. they interact with one another and their interaction gives shape to behaviour, thoughts and emotions.) Abnormal symptoms are viewed as the result of conflicts between these forces.	) This model was first formulated by Freud who believed that three components shape personality — instinctual needs, drives and impulses (id), rational thinking (ego), and moral standards (superego).) Freud stated that abnormal behaviour is a symbolic expression of unconscious mental conflicts that can be generally traced to early childhood or infancy.
Behavioural Model	Psychological	This model states that both normal and abnormal behaviours are learned and psychological disorders are the result of learning maladaptive ways of behaving.	) The model concentrates on behaviours that are learned through conditioning and proposes that what has been learned can be unlearned.) Learning can take place by: - Classical conditioning (temporal association in which two events repeatedly occur close together in time) - Operant conditioning (behaviour is followed by a reward) - Social learning (learning by imitating others' behaviour)
Cognitive Model	Psychological	This model states that abnormal functioning can result from cognitive problems.	) People may hold assumptions and attitudes about themselves that are irrational and inaccurate.) People may also repeatedly think in illogical ways and make overgeneralisations, that is, they draw broad, negative conclusions on the basis of a single insignificant event.

Humanistic-Existential Model	Psychological	) This model focuses on broader aspects of human existence.	) <u>Humanists</u> believe that human beings are born with a natural tendency to be friendly, cooperative and constructive, and are driven to self-actualise, i.e. to fulfil this potential for goodness and growth.
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			2) <u>Existentialists</u> believe that from birth we have total freedom to give meaning to our existence or to avoid that responsibility. Those who shirk from this responsibility would live empty, inauthentic, and dysfunctional lives.
Socio-Cultural Factors	Socio-cultural	) Abnormal behaviour is best understood in light of the social and cultural forces that influence an individual.) As behaviour is shaped by societal forces, factors such as family structure and communication, social networks, societal conditions, and societal labels and roles become more important.	) It has been found that certain family systems are likely to promote abnormal functioning in individuals and members.) Some families have an enmeshed structure in which the members are over involved in each other's activities, thoughts, and feelings. Children in this kind of family may have difficulties in becoming independent in life.) The broader social networks in which people operate include their social and professional relationships.) Studies have shown that people who are isolated and lack social support, i.e. strong and fulfilling interpersonal relationships in their lives are likely to become more depressed and remain depressed longer than those who have good friendships.) Socio-cultural theorists also believe that abnormal functioning is influenced by the societal labels and roles assigned to troubled people. A person gradually learns to accept and play the sick role, and functions in a disturbed manner.

Diathesis-Stress Model	Diathesis-stress	This model states that psychological disorders develop when a diathesis (biological predisposition to the disorder) is set off by a stressful situation.	1) This model has three components. 2) The first is the diathesis or the presence of some biological aberration which may be inherited. 3) The second component is that the diathesis may carry a vulnerability to develop a psychological disorder. This means that the person is at 'risk' or 'predisposed' to develop a disorder. 4) The third component is the presence of pathogenic stressors, i.e. factors/stressors that may lead to psychopathology. If such "at risk" persons are exposed to these stressors, their predisposition may actually evolve into a disorder.
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Types of Disorders

1. **Anxiety Disorders**

Anxiety is usually defined as a diffuse, vague, very unpleasant feeling of fear and apprehension.

Symptoms are: rapid heart rate, shortness of breath, diarrhoea, loss of appetite, fainting, dizziness, sweating, sleeplessness, frequent urination and tremors.

Anxiety is of many types:

1. **Generalized anxiety disorder:** this consists of prolonged, vague, unexplained and intense fear which are **not attached to any particular object or person**. **Symptoms: worry, apprehensive feeling about future, hypervigilance** that involves **constantly scanning the environment for dangers**.
2. **Panic disorder: recurrent anxiety attacks**, feeling **intense terror**. A panic attack denotes an abrupt of intense anxiety rising to a peak when thoughts of a **particular stimuli are present**.

Symptoms: shortness of breath, dizziness, trembling, palpitations, choking, nausea, chest pain or discomfort, fear of going crazy, losing control or dying.

3. **Phobias:** People with phobia have irrational fear related to a specific object, people or situations. Phobia often develops gradually, starts with a general anxiety disorder. E.g. people scared to climb heights, afraid of deep waters, in an elevator. Can be grouped **into 3 types**.

Specific phobia, social phobia, agoraphobia.

Specific Phobia– most common. **Includes irrational fears of a certain animal, or being in an enclosed space.**

Social Phobia– **Intense and incapacitating** fear and embarrassment when **dealing with other characters**.

Agoraphobia– is when people develop a fear of entering **unfamiliar situations**. Or sometimes afraid of entering their home.

4. **Obsessive compulsive disorder**- unable to control their preoccupation with specific ideas or unable to prevent themselves from repeating a particular act/series of acts that affect their ability to carry out normal activities.

Obsessive: is when you cannot stop thinking about an idea or a topic, generally unpleasant or shameful.

Compulsive: is the urge to repeat/perform certain behaviours/acts over and over again. E.g. counting a bundle, order checking, washing hands repeatedly.

5. **Post-traumatic stress disorder**- Very rarely when some people have been caught in a natural disaster, floods, earthquakes, tsunami, war, terrorist attack, major accident etc.

experience this disorder.

Symptoms- dreams, flashbacks, impaired concentration and emotional numbing.

Somatoform Disorder:

These are conditions, where there are physical symptoms despite no/absence of any physical disease.

In this the individual has physical difficulties but complains of physical symptoms, for which there is no biological cause.

These disorders include:

Pain Disorder, somatization, conversion and hypochondriasis

Pain Disorder- report extreme and incapacitating pain without any identifiable biological symptoms.

There are two methods to address this:

1. **Active coping**– Remaining active and ignoring the pain.
2. **Passive coping**– Reduced activity and social withdrawal.

Somatization disorder- people with multiple, recurrent or chronic bodily complaints. They tend to present their complaints in an **exaggerated or dramatic way**.

e.g. headaches, fatigue, heart palpitation, fainting spells, vomiting and allergies. People with this disorder believe that they are sick, provide long history of their illness and consume large quantities of medicines.

Conversion disorder- symptoms are **reported loss of part or all of some basic body functions**.

Paralysis, blindness, deafness and difficulty in walking are the generally reported symptoms **after a sudden stressful/traumatic experience**.

Hypochondriasis- is diagnosed if a person has a persistent belief that s/he is **suffering from a serious illness, despite the medical reassurance, lack of physical findings**. Hypochondriacs have an obsessive preoccupation and concern with the condition of their bodily organs and they continuously worry about their health.

Dissociative Disorders-

Dissociation can be viewed as **severance** of the connections **between ideas and emotions**. It involves **feeling** of unreality, **estrangement, depersonalization** and loss or shift of identity. They are of 4 categories:

1. **Dissociative Amnesia**: Extensive but selective memory loss that has no known organic cause like head injury or anything due to overwhelming stress.

Symptoms- inability to remember the past or specific people, events, places, names.

1. **Dissociative fugue**: due to unexpected travel away from home or workplace, people sometimes assume new identity and inability to recall the previous identity. The fugue usually ends when person returns to regular life or wakes up.
2. **Dissociative identity disorder**: often referred to as **multiple personalities** is the most dramatic of these disorders. It is often associated with traumatic experiences in childhood.

3. **Depersonalization:** Involves a **dreamlike state** in which the person has a sense of being **separated from self** and reality both.

Mood Disorder

Disturbance in mood or prolonged emotional state. Main type of mood disorders include: depressive, manic and bipolar disorders.

1. **Depressive disorder:** The most common mood disorder is depression. Factors that disposition towards depression could be majorly genetic, heredity. Age and gender are also the factors. Eg. Women are more susceptible during young adulthood and men during middle age.

Symptoms: Depicts depressed mood or loss of interest or pleasure in most activities, together with symptoms which include change in body weight, constant sleep problems, tiredness, inability to think clearly, agitation, greatly slowed behaviour. At times even thoughts of death and suicide. Feeling of worthlessness or excessive guilt.

1. Mania: Maniac episodes rarely appear by themselves, they usually alternate with depression.

Symptoms: People suffering from mania become euphoric, extremely active, excessively talkative and easily distractible.

1. Bipolar: When mania and depression alternately appear, sometimes they are interrupted by periods of normal mood. This is referred as bipolar mood disorder, earlier referred as maniac – depressive disorder.

Symptoms: Suicide attempt is the highest. Several risk factors influence this behaviour- mental disorder, age, gender, ethnicity (Japanese), recent occurrence of serious life event.

Teenagers and those over 70 years are more at risk. Men contemplate suicide attempt more than women.

Q: How can suicides be prevented?

A: Suicides can be prevented by staying alert to some of the symptoms which include:

- Changing in eating and sleeping habits.
- Withdrawal from friends, family and regular activities.
- Violent actions, rebellious behaviour, running away.
- Drug and alcohol
- Marked personality change
- Persistent boredom
- Difficulty in concentration
- Complaints about physical symptoms
- Loss of interest in pleasurable activities.

Paying attention to such symptoms and seeking timely help by professional counsellor/psychologist can help to prevent the likelihood of suicide.

Schizophrenia Disorders

Schizophrenia is the descriptive term for a group of psychotic disorders in which personal, social and occupational functioning deteriorates as a result of disturbed thought processes, strange perceptions, unusual emotional states and motor abnormalities.

Symptoms: can be grouped into 3 categories- Positive (i.e. excesses of thought, emotion and behaviour), Negative (deficit of thought, emotion and behaviour) and psychomotor symptoms.

1. **Positive Symptoms:** pathological excesses or bizarre additions to a person's behaviour.

Symptoms: Delusions, disorganized thinking and speech, heightened perception and hallucinations.

Q: What are Delusions:

Delusions – false belief that is firmly held on inadequate grounds.

Delusions of persecution- They believe plotted against, spied on, slandered, threatened, attacked or deliberately victimized.

Delusions of reference- in which they attach personal meaning to the actions, objects and events

Delusions of grandeur-They believe themselves to be specially empowered.

Delusions of control- They believe their thoughts, feelings and actions are controlled by others.

Q: What are Hallucinations?

A: Perceptions that occur in absence of external stimuli.

They are of several types-

Auditory Hallucinations: Patients hear sounds or voices that speak directly to them.

Tactile Hallucinations: Tingling, burning sensations.

Somatic: Something happening inside the body, such as snake crawling inside stomach.

Visual: Distinct visions of people and objects.

Gustatory: Food or drink taste strange.

Olfactory: Smell of poison or smoke.

1. **Negative symptoms:** are pathological deficits and include **Alogia** (reduction in speech content or poverty of speech), **Blunted effect**– Less expression of sadness, joy, anger and other feelings.

Flat effect- No emotions and feelings

Loss of volition- Apathy or inability to start or complete any work.

Social withdrawal- become focused on their own ideas and fantasies.

1. **Psychomotor symptoms:** Less spontaneous, make odd grimaces and gestures. Types:

Catatonic stupor: remain motionless and silent for long durations.

Catatonic rigidity: maintain a rigid upright posture for hours.

Catatonic posturing: assuming odd, awkward positions.

Behavioural and Developmental disorders:

These disorders are specific to children and if neglected can lead to serious consequences later in life.

Why Children: Children have less self-understanding and they have not developed a stable sense of identity. Nor do they have adequate frame of reference regarding reality, possibility and value. As a result they cannot cope with the stressful events effectively and hence it reflects in their behaviour and impacts their development.

Other childhood disorders:

Pervasive Developmental Disorders: in addition children may also suffer from more serious developmental disorder called Pervasive Developmental Disorders.

Autistic Disorder (Autism)- These children have marked difficulties in social interaction, sharing emotions, communication skills, interests in activities. 70% of autistic children have chances of being mentally retarded. Such children may reflect repetitive behaviours such as lining up objects or stereotyped body movements such as rocking. These movements are self stimulatory.

Eating Disorder:

Anorexia nervosa– Individual has a distorted body self-image and may think s/he is overweight. Hence avoid eating, starve and over exercise compulsively to lose weight.

Bulimia Nervosa; Individuals eat excessive amounts of food. Then purge with help of eating medicines like laxatives or diuretics or by vomiting.

Binge eating: There are frequent episodes of out of control eating.

Substance use disorders:

Substance dependence – Intense craving, withdrawal symptoms, compulsive drug/alcohol intake.
Substance abuse- There are recurrent and adverse consequences of taking these substances and damage their social, family relationships and performance at work.

Alcohol impact:

- All alcohol beverages contain Ethyl Alcohol.
- This chemical is absorbed in the blood and carried into central nervous system and spinal cord.
 - It depresses those areas in brain that control judgement, inhibition.
 - People become talkative, friendly, lose inhibitions and feel more confident and happy.
 - They also become more emotional, loud and aggressive.
- Speech becomes unclear, memory falters and physical movements can become unsteady. Therefore drinking and driving is not allowed under laws.

Intellectual disability: People with IQ less than 70 show deficit or impairment in adaptive behaviour. Lower functional, academic skills.

Chapter 5- Therapeutic Approaches

Q1. Describe the nature and scope of Psychotherapy. Highlight the importance of therapeutic approaches in a psychotherapy (2 Marks)

Ans. Psychotherapy is a voluntary relationship between the one seeking treatment or the client and the one who treats or the therapist.

1. Purpose: To help the client to solve the psychological problems being faced by her or him.
2. Aim: To change the maladaptive behaviors, decrease the sense of personal distress, and help the client to adapt better to his/her environment.
3. The relationship is conducive for building the trust of the client so that problems may be freely discussed.

Q2. What are the characteristics of a psychotherapy? (2/3/4 Marks)

Ans.

1. There is systematic application of principles underlying the different theories of therapy.
2. Only persons who have received practical training under expert supervision can practise psychotherapy.
3. The situation involved a therapist and client who seeks and receives help for his/her emotional problems (this person is the focus of attention in the therapeutic process).
4. The interaction of the therapist and the client results in the consolidation or formation of the therapeutic relationship. This is a confidential, interpersonal, and dynamic relationship.

Q3. What are the goals of psychotherapy? (2/3/4 Marks)

Ans. Reinforcing client's resolve for betterment.

- (ii) Lessening emotional pressure.
- (iii) Unfolding the potential for positive growth.
- (iv) Modifying habits.
- (v) Changing thinking patterns.
- (vi) Increasing self-awareness.
- (vii) Improving interpersonal relations and communication.
- (viii) Facilitating decision-making.
- (ix) Becoming aware of one's choices in life.
- (x) Relating to one's social environment in a more creative and self-aware manner. Therapeutic Relationship:

The special relationship between the client and the therapist is known as the therapeutic relationship or alliance.

Q4. What are the components of a psychotherapeutic approach

Ans. **Components:**

1. Contractual Nature of the Relationship: Two willing individuals, the client and the therapist, enter into a partnership which aims at helping the client overcome his/ her problems.
2. Limited Duration of the Therapy: This alliance lasts until the client becomes able to deal with his/her problems and take control of his/her life.

Properties:

- (i) It is a trusting and confiding relationship.

- (ii) The high level of trust enables the client to unburden herself/himself to the therapist and confide her/his psychological and personal problems to the latter.
- (iii) The therapist encourages this by being accepting, empathetic and warm to the client. (iv) Maintaining a non-judgmental attitude towards the client.
- (v)- Unconditional Positive Regards-
- (vi)- Empathy- Understanding the plight of another person and accepting the other person. It is basically understanding things from other's perspective
- (vii)- The therapist must keep strict confidentiality of the experiences and events. (viii)-The therapist must not exploit the trust and confidence of the other person

Q. Briefly discuss the parameters on which the psychotherapies is based. (4/6 Marks)

Parameter	Psychodynamic	Cognitive & Behavioral	Humanistic
Cause	Intra Psychic conflicts Present within the psyche of the person are the cause of intrapsychic conflicts	Psychological problems arise due to faulty learning of behavior and Cognition	Questions about meaning of existence is the cause of intrapsychic conflicts
Cause come into existence	Unfulfilled desires of childhood	Faulty learning, faulty cognitions	The current feelings of loneliness and alienation, sense of futility in one's existence
Method of treatment	Free Association Dream analysis	Identification of faulty cognitions, learning and alternate behavior contingencies to improve the behavior	Existential provides an environment which is warm. Positive, accepting and non judgmental
Nature of relationship between the therapist and the client	The therapist understands the nature of the intrapsychic conflict better than the client	The therapist is able to understand the nature of the faulty learning and behavior problems and replaces the irrational thought and faulty learning with a more rational approach	The therapist provides a warm and an empathetic relationship to the client in which the client feels secure and can explore the causes of his/her problems.
Chief Benefit to the Client	Emotional Insight when the client understands the problem intellectually and deal with it emotionally	changing the maladaptive behavior pattern. Learning more adaptive form of behavior patterns, reduction of distress	Humanistic therapy values personal growth as a chief benefit to the client. Personal growth is the process of understanding one's

			aspirations, emotions and motives
Duration of therapy	Several years	Few months	Few months

BEHAVIOUR THERAPY- 6 Marks (Very important)

- Focused on the behaviour and thoughts of the client in the present. Faulty learning arises due to faulty behavior patterns or learning
- The past is relevant only to the extent of understanding the origins of the faulty behaviour and thought patterns, not relieved.
- Behaviour therapies are clinical application of learning theories. (Classical conditioning, operant conditioning and observational learning)
- Consists of a large set of specific techniques and interventions—symptoms of the client and the clinical diagnosis are the guiding factors in the selection of the specific techniques or interventions to be applied.

The foundation of behavior therapy is on formulating dysfunctional/ faulty behavior patterns, the factors which reinforce and maintain this behavior

Method of Treatment:

- (i) The client is interviewed with a view to analyse his/her behavior patterns.
- (ii) Behavioural analysis is conducted to find:
 - (a) **Malfunctioning Behaviors:** Behaviours which cause distress to the client.
 - (b) **Antecedent Factors:** Those causes which predispose the person to indulge in that behaviour
 - (c) **Maintaining Factors:** Those factors which lead to the persistence of the faulty behaviour.
- (iii) **Aim:** To eliminate the faulty behaviours and substitute them with adaptive behaviour patterns.
 - (a) **Antecedent Operations:** Control behaviour by changing something that precedes such a behaviour.
 - (b) **Establishing Operations:** Induce a change in behaviour by increasing or decreasing the reinforcing value of a particular consequence.
(give an example to explain the phenomenon)
 - (c) Consequent Operation: i.e., Giving reinforcement eg. Praise.

Behavioural Techniques:

1. **Negative Reinforcement:** Following an undesired response with an outcome that is painful or not liked.
2. **Aversive Conditioning:** Repeated association of undesired response with an aversive consequence present reality.
3. **Positive Reinforcement:** Given to increase the deficit if an adaptive behaviour occurs rarely.
4. **Token Economy:** Give a token as a reward every time a wanted behaviour occurs, which can be collected and exchanged for a reward.
5. **Differential Reinforcement:** Unwanted behaviour can be reduced (negative reinforcement) and wanted behaviour (positive reinforcement) can be increased simultaneously.
The other method is to positively reinforce the wanted behaviour and ignore the unwanted behaviour—less painful and equally effective.
6. **Systematic Desensitization:** A technique introduced by Wolpe for treating phobias or irrational fears. – VERY IMPORTANT
 - (i) The client is interviewed to elicit fear provoking situations.
 - (ii) With the client, the therapist prepares a hierarchy of anxiety—provoking stimuli with the least anxiety-provoking stimuli at the bottom.
 - (iii) The therapist relaxes the client and asks the client to think about the least anxiety-provoking situation.
 - (iv) The client is asked to stop thinking of the situation if tension is felt.
 - (v) Over sessions, the client is able to imagine more severe fear provoking situations while maintaining the relaxation.
 - (vi) The client gets systematically desensitized to the fear.

Operates on the principle of reciprocal inhibition—the presence of two mutually opposing forces (relaxation response vs. anxiety-provoking scene) at the same time, inhibits the weaker force.

The client is able to tolerate progressively greater levels of anxiety because of his/her relaxed state.

Question Bank

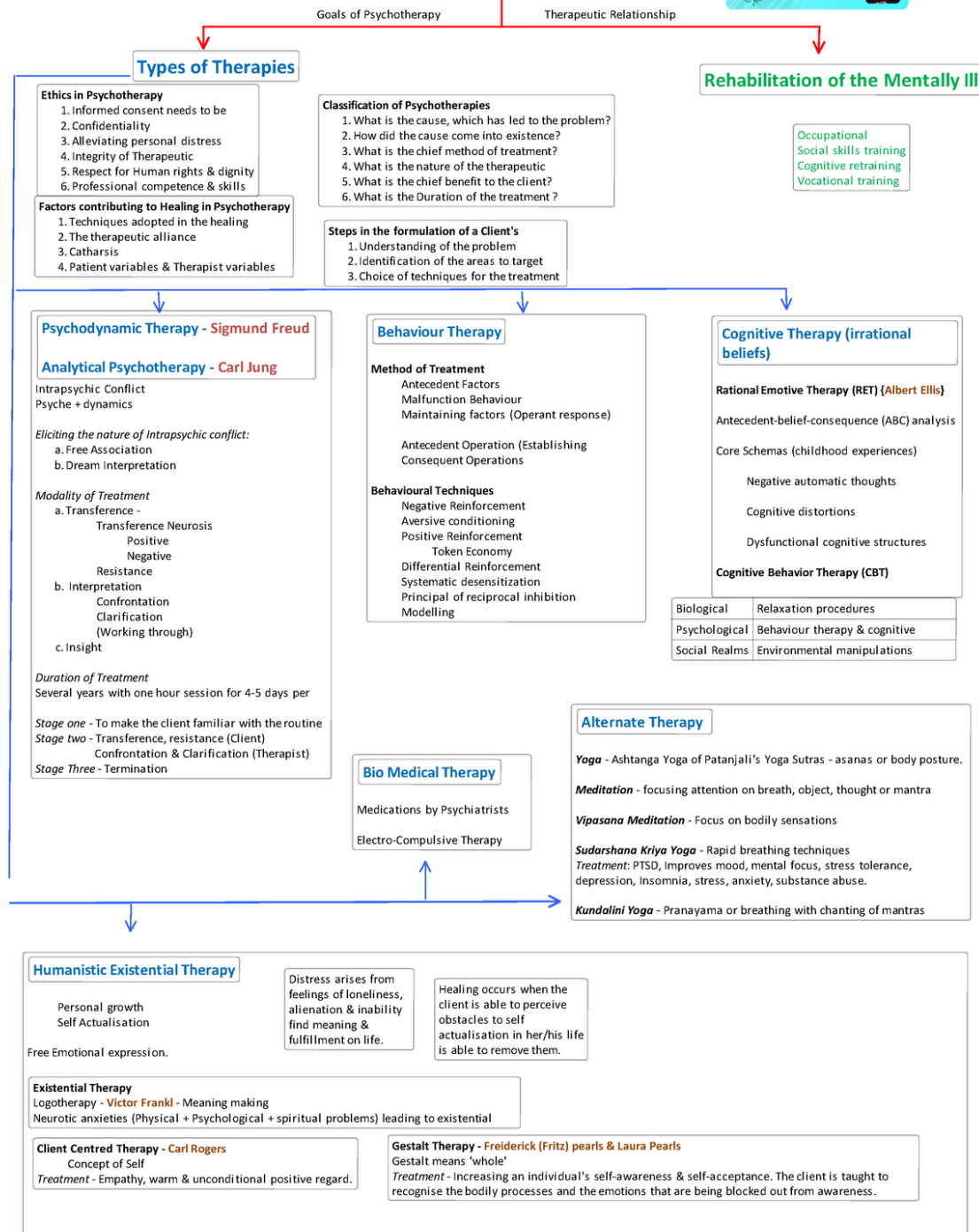
1. Discuss various techniques used in behavior therapy- write only the techniques
2. Discuss basic principles of behavior therapy (6 Marks)- The complete answer

3. What is systematic desensitization. What are the various steps involved in systematic desensitization.
4. What is reinforcement? Discuss its subtypes.
5. What kind of therapy is best suited for treating phobia for lizards and cockroaches?
How can the concept of reinforcement be used as a behavior modification technique

Therapeutic Approaches

Saturday, October 8, 2022
10:56 AM

Psychotherapy



C. COGNITIVE THERAPY

1. Rational Emotive Therapy (RET) (Albert Ellis):

- Irrational beliefs mediate between the antecedent events and their consequences.
- The first step in RET is the antecedent-belief-consequence (ABC) analysis. Antecedent events, which caused the psychological distress, are noted.
 - (ii) Client is interviewed to find out irrational beliefs, which distort the reality
 - (iii) The therapist encourages this by being accepting, empathic, genuine and warm to the client.
 - (iv) The therapist conveys by his/her words and behaviours that he/she is not judging the client and will continue to show the same positive feelings towards the client even if the client is rude or confides all the 'wrong' things that he/she may have done or thought about. This is the unconditional positive regard which the therapist has for the client.

The clinical formulation is an ongoing process. Formulations may require reformulations as clinical insights are gained in the process of therapy. Distorted perception of the antecedent event due to the irrational belief leads to the consequence, i.e., negative emotions and behaviours.

- Non-directive questioning: Process by which irrational beliefs are refuted by the therapist.
 - (i) Nature of questioning is gentle, without probing or being directive.
 - (ii) Make the client think deeper into his/her assumptions about life and problems.
- Client changes the irrational beliefs by making a change in his/her philosophy about life—rational belief system replaces the irrational belief system.

2. Aaron Beck's Cognitive Therapy:

- (i) Childhood experiences provided by the family and society develop **core schemas** or systems, which include beliefs and action patterns in the individual.
- (ii) Critical events in the individual's life trigger the core, leading to the development of negative automatic thoughts.
- (iii) Negative thoughts are persistent irrational thoughts characterized by cognitive distortions.
- (iv) Dysfunctional Cognitive Structures: Patterns of thought which are general in nature but which distort the reality in a negative manner.
- (v) Repeated occurrence of these thoughts leads to the development of feelings of anxiety and depression.
- The therapist uses questioning, which is gentle, non-threatening disputation of the client's beliefs and thoughts.
- The questions make the client think in a direction opposite to that of the negative automatic thoughts whereby she/he gains insight into the nature of her/his dysfunctional schemas, and is able to alter her/his cognitive structures.

3. Cognitive Behaviour Therapy (CBT):

- Short, comprehensive, effective treatment for a wide range of psychological disorders such as anxiety, depression, panic attacks and borderline personality.
- Adopts a biopsychosocial approach to the delineation of psychopathology.
- Combines cognitive therapy with behavioural techniques.
- Rationate—distress has its origins in the biological, psychological, and social realms.
- Addresses the biological (relaxation procedures), psychological (behaviour and cognitive therapy) and social (environmental manipulations) aspects.

Question Bank

1. Explain with the help of an example how does cognitive distortions takes place.
2. What kind of problems is cognitive behavior problems best suited for?
3. Riya is unable to clear her premedical examination. She is in a state of depression and feels that she is good for nothing. Using cognitive behavior techniques how will you rationalize her irrational thoughts.

D. Humanistic-Existential Therapy- Carl Roger, Maslow, Victor Frankl- VERY IMPORTANT

BASIC PREMISE: Psychological distress arise from the feeling of loneliness, alienation and inability to find meaning and fulfilment in one's life.

Human beings are motivated by the desire for growth and self-actualization.

When needs are curbed by the society the family experience psychological distress.

Self-actualization is defined as an innate force that moves the person to become more complex, balanced, and integrated; integrated means a sense of whole, being a complete person.

1. Self-actualization requires free emotional expression:

- (a) The family and society curb emotional expression, as it is feared that a free expression of emotions can harm society by unleashing destructive forces.
 - (b) When emotionally expression is curbed, destructive behaviour and negative emotions by thwarting the process of emotional integration.
2. Healing occurs when the client is able to perceive the obstacles to self-actualization in his/her life and is able to remove them.

Role of the therapist:

3. Therapy creates a permissive, non-judgmental and accepting atmosphere in which the client's emotions can be freely expressed.
4. The client has the freedom and responsibility to control his/her own behavior; the therapist is merely a facilitator and guide. The chief aim of the therapy is to expand the client's awareness. Healing takes place by the process of understanding the unique personal experience of the client



1. Existential Therapy [Logotherapy (Victor Frankl)]:

- Treatment for the soul.
- Meaning making: Process of finding meaning even in life-threatening circumstances, the basis of which is a person's quest for finding the spiritual truth of one's existence.
- Spiritual Unconscious: The storehouse of love, aesthetic awareness and values of life.
- Existential Anxiety: Neurotic anxiety of spiritual origin (spiritual anxieties leading to meaninglessness).
- Goal: To help the patients find meaning and responsibility in their life irrespective of their life circumstances.
- The therapist emphasizes the unique nature of the patient's life and is open (shares his/her feelings, values and own existence).
- Emphasis is on here and now, the therapist reminds the client about the immediacy of the present.

2. Client-centered Therapy (Carl Rogers):

- Introduced the concept of self and freedom and choice as the core of one's being.
 - Provides a warm relationship in which the client can reconnect with his/her disintegrated feelings.
 - The therapist:
 - (i) Shows empathy—understands the client's experience as if it were his/her own—sets up an emotional resonance between client and therapist.
 - (ii) Warmth—the client feels secure and can trust the therapist.
 - (iii) Has unconditional positive regard, i.e., total acceptance of the client as he/she is, indicates that the positive warmth of the therapist is not dependent on what the client reveals or does in the therapy sessions.
 - Client feels secure enough to explore his/her feelings; therapist reflects the feelings of the client in a non-judgemental manner the reflection is achieved by rephrasing the statements of the client, i.e., seeking simple clarifications to enhance the meaning of the client's statements.
- Reflection is achieved by rephrasing the statements of the client. The process of reflection helps the client to become more integrated.

Role of the therapist is that of a facilitator

3. Gestalt Therapy (Frederick and Laura Pearl):

- Goal: To increase an individual's self-awareness and self-acceptance.
- Client is taught to recognize the bodily processes and the emotions that are being blocked out from awareness.
- Therapist encourages the client to act out fantasies about feelings and conflicts can also be used in group settings.
- (i) What are the factors that contribute towards healing of psychotherapy

Factors Contributing to Healing: (4 Marks)- important

1. TECHNIQUES ADOPTED BY THE THERAPIST AND THE IMPLEMENTATION OF THE SAME WITH THE

CLIENT: CBT for an anxious client—relaxation procedures and cognitive restructuring contribute to the healing.

2. THE THERAPEUTIC ALLIANCE, which is formed between the therapist and the patient/ client, has healing properties, because of the regular availability of the therapist, and the warmth and empathy provided by the therapist.

3. CATHARSIS: A process of emotional unburdening by a client when he/she is being interviewed in the initial sessions of therapy to understand the nature of the problem.

4. NON-SPECIFIC FACTORS: These factors occur across different systems of psychotherapy and across different clients/patients and different therapists.



(I) PATIENT VARIABLES (motivation for change, expectation of improvement).

(II) THERAPIST VARIABLES (positive nature, good mental health, absence of unresolved emotional conflicts).

What are the Ethical considerations in a Psychotherapy: (2 Marks)

1. Informed consent needs to be taken.
2. Confidentiality of the client should be maintained.
3. Alleviating personal distress should be the goal of all attempts of the therapist.
4. Integrity of the practitioner-client relationship is important.
5. Respect for human rights and dignity.
6. Professional competence and skills are essential.

. ALTERNATIVE THERAPIES Yoga:

- An ancient Indian technique detailed in the Ashtanga Yoga of Patanjali's Yoga Sutras.
 - Refers to only the asanas (body posture component) or to pranayama (breathing practices).
 - Techniques enhance well-being, mood, attention, mental focus, and stress tolerance.
 - Reduces the time to go to sleep and improves the quality of sleep.
 - Proper training by a skilled teacher and 30-minute practice everyday maximises the benefits.
- Meditation refers to the practice of focusing attention on breath or on an object or thought of a mantra.

Sudarshana Kriya Yoga (SKY)

- (i) Rapid breathing techniques induce hyperventilation.
- (ii) Beneficial, low risk, low cost.
- (iii) Used as a public health intervention technique to alleviate PTSD in survivors of mass disasters.
- (iv) Reduces depression (research conducted at the National Institute of Mental Health and Neurosciences (NIMHANS)).
- (v) Reduces stress levels in substance abuse patients, e.g., alcoholics.

B. Kundalini Yoga

- (i) Effective in treatment of mental disorders and OCD.

CH-6 ATTITUDES AND SOCIAL COGNITION

Definition

Attitude is a state of the mind, a set of views or thoughts, regarding some topic (called the 'attitude object'), which have an evaluative feature (positive, negative or neutral quality).

A-B-C Components of an Attitude:

Affective- emotional component **Behavioral** tendency to act **Cognitive-** thought component

Beliefs

Beliefs refer to the cognitive component of attitudes and form the ground on which attitudes stand, such as belief in God, or belief in democracy as a political ideology.

Values

- Values are attitudes or beliefs that contain a 'should' or 'ought' aspect, such as moral or **ethical values**. One example of a value is hard work or honesty.

- Values are formed when a particular belief or attitude becomes an inseparable part of the person's outlook on life. Consequently, **Values are different to change.**

Features of Attitude

Four significant features of attitudes are as follows:

1. Valence (positivity or negativity):

- The valence of an attitude tells us whether an attitude is positive or negative towards the attitude object.
- 2. **Extremeness:** The extremeness of an attitude indicates how positive or negative an attitude is. On a scale of 5, rating of 1 and 5 are extreme, ratings of 2 and 4 are less extreme, and 3 is neutral.

3. Simplicity or Complexity (multiplexity): This feature refers to how many attitudes there are within a broader attitude. Think of an attitude as a family containing several 'member'

attitudes. · Simple Attitude - contains only one or a few attitudes.

- Complex Attitude - Is made up of many attitudes.

4. Centrality: This refers to the role of a particular attitude in the attitude system. An attitude with greater centrality would influence the other attitudes in the system much more than non- central (or peripheral) attitudes would.

ATTITUDE FORMATION

- **Process of Attitude Formation** – There are various processes which lead to attitude formation, some of them are as follows:-
 - a. **Learning attitudes by association:** You might have seen that students often develop a liking for a particular subject because of the teacher. This is because they see many positive qualities in that teacher; these positive qualities get linked to the subject that s/he teaches, and ultimately get expressed in the form of liking for the subject. In other words, a positive attitude towards the subject is learned through the positive association between a teacher and a student.
 - b. **Learning attitudes by being rewarded or punished:** If an individual is praised for showing a particular attitude, chances are high that s/he will develop that attitude further. And he/she is punished then he will develop a negative attitude towards that thing.



- c. **Learning attitudes through modelling (observing others):** We learn attitudes by observing others being rewarded or punished for showing behaviour of a particular kind towards the attitude object. For example, children may form a respectful attitude towards elders, by observing that their parents show respect for elders, and are appreciated for it.
- d. **Learning attitudes through group or cultural norms:** Norms are unwritten rules about behaviour that everyone is supposed to show. When individuals see that a particular behaviour is shown by others in society, which is socially approved, they may ultimately develop a positive attitude towards such behaviour.
- e. **Learning through exposure to information:** Today, with the huge amount of information that is being provided through various media, both positive and negative attitudes are being formed

FACTORS THAT INFLUENCE ATTITUDE CHANGE:

- a. **Family and School Environment:** Particularly in the early years of life, parents and other family members play a significant role in shaping attitude formation. Later, the school environment becomes an important background for attitude formation.
- b. **Reference Groups-** are groups that individual joins voluntarily for eg. A political party, a dance group in school etc. These groups have certain norms which facilitate the process of attitude formation towards a topic.
- c. **Personal Experiences:** Many attitudes are formed through direct personal experiences which bring about a drastic change in our life. For e.g a Man who was the only survivor in an accident, developed a positive attitude towards helping others and worked for the betterment of the society after his accident.
- d. **Media-related Influences:** Internet very powerful sources of information that lead to attitude formation and change. In addition, school level textbooks also influence attitude formation. These sources first strengthen the cognitive and affective components of attitudes, and subsequently may also affect the behavioural component.

ATTITUDE CHANGE

Attitudes that are still in the formative stage, and are more like opinions, are much more likely to change compared to attitude that have become firmly established and have become a part of the individual's values.

Process of Attitude Change:

1. **Balance or P-O-X triangle (Fritz Heider) represents the relationships between three aspects or components of attitude:**

- **P** is the person whose attitude is being studied,
- **O** is another person
- **X** is the topic towards which the attitude is being studied (attitude object).

It is also possible that all three are persons. The basic idea is that an attitude changes if there is a state of imbalance between the **P-O attitude**, **O-X attitude**, and **P-X attitude**. This is because imbalance is logically uncomfortable.



Condition of Imbalance:

- All three sides of POX are **negative**,
- or **two** sides are **positive**, and **one** side is **negative**.

Condition of Balance:

- All three sides of POX are **positive**
- or **two** sides are **negative**, and **one** side is **positive**.

For example of **dowry as an attitude topic (X)**. Suppose a person (P) has a positive attitude towards dowry (P-X positive). P is planning to get his son married to the daughter of **some person (O)** who has a negative attitude towards dowry (O-X negative). What would be the nature of the P-O attitude, and how would it determine balance or imbalance in the situation? If O initially has a positive attitude towards P, the situation would be unbalanced. P-X is positive, O-P is positive, but O-X is negative.

That is, there are two positives and one negative in the triangle. This is a situation of imbalance. One of the three attitudes will therefore have to change. This change could take place in the P-X relationship (P starts disliking dowry as a custom), or in the O-X relationship (O starts liking dowry as a custom), or in the O-P relationship (O starts disliking P). In short, an attitude change will have to take place so that there will be three positive relationships, or two negative and one positive relationship, in the triangle

b. Concept of Cognitive Dissonance by (Leon Festinger)

- The basic idea is that the cognitive components of an attitude must be 'consonant' i.e., they should be logically in line with each other. If an individual finds that two cognitions in an attitude are dissonant, then one of them will be changed in the direction of consonance.
- For example **Cognition I**: Pan masala causes mouth cancer which is fatal.
- **Cognition II**: I eat pan masala. Holding these two ideas or cognitions will make any individual feel that something is 'out of tune', or dissonant, in the attitude towards pan masala. Therefore, one of these ideas will have to be changed, so that consonance can be attained.

Cognitive Consistency:

Cognitive consistency means that two components, aspects or elements of the attitude, or attitude system, must be in the same direction. Each element should logically fall in line with other elements.

c. Two Step Concept (By S.M Mohsin):

According to him, attitude change takes place in the form of two steps.

- In the first step, **the target of change identifies with the source. The 'target' is the person whose attitude is to be changed. The 'source' is the person through whose influence the change is to take place.** Identification means that the target has liking and regard for the source.
- In the second step, the **source herself/himself shows an attitude change. Observing the source's** changed attitude and behaviour, **the target also shows an attitude change** through behaviour. This is a kind of imitation or observational learning

Factors that Influence Attitude Change

a. Characteristics of the existing attitude:

All four properties of attitudes mentioned earlier, namely:-

- **Valence** (positivity or negativity)- positive attitudes are easier to change than negative attitudes are.
- **Extremeness**- Extreme attitude difficult to change
- **Simplicity or Complexity** (Multiplexity)-Complex attitude are difficult to change the a simple one.
- **Centrality**- its difficult to change a central attitude. An attitude change may be **congruent**

— it may change in the same direction as the existing attitude. An attitude change may be **in-congruent** — it may change in a direction opposite to the existing attitude. It has been found that, in general, congruent change

- b. **Source characteristics:** Source **credibility** and **attractiveness** are two features that affect attitude change. Attitudes are more likely to change when the message comes from a highly credible source rather than from a low credible source. For example, adults who are planning to buy a laptop are more convinced by a computer engineer than they would be by a schoolchild. In the case of some products such as cars, sales may increase if they are publicized, not necessarily by experts, but by popular public figures.

c. Message characteristics:

- Attitudes will change when the **amount of information** that is given about the topic is just enough, neither too much nor too little.
- Whether the **message contains a rational or an emotional appeal** also makes a difference.
- The **motives activated by the message also determine attitude change**. For example, drinking milk may be said to make a person healthy and good-looking, or more energetic and more successful at one's job

- d. **Target characteristics:** Following Qualities of the target influence the likelihood and extent of attitude change:-

- **Personality**- People, who have a more open and flexible personality, change more easily.
- **Prejudice**- People with strong prejudices are less prone to any attitude change.
- **Self- Esteem**- People who have a low self-esteem, and do not have sufficient confidence in themselves, change their attitudes more easily.
- **Intelligence**- More intelligent people may change their attitudes less easily than those with lower intelligence.

Attitude-Behaviour Relationship:

There would be consistency between attitudes and behaviour when:

- the attitude is strong, and occupies a central place in the attitude system,
- the person is aware of her/his attitude,
- There is very little or no external pressure for the person to behave in a particular way.
- the person's behaviour is not being watched or evaluated by others, and
- the person thinks that the behaviour would have a positive consequence

PREJUDICE AND DISCRIMINATION:

- **Stereotypes (the cognitive component)**- is a **cluster of ideas regarding the characteristics of a specific group**. All members belonging to this group are assumed to possess these characteristics.
- **Prejudices (Affective Component)**- Often, stereotypes consist of **undesirable characteristics about the target group, and they lead to negative attitudes or prejudices towards members of specific groups**.



- **Discrimination (Behavioural Component)- people behave in a less positive way towards a particular target group compared to another group** which they favour. For e.g. the genocide committed by the Nazis in Germany against Jewish people.

Prejudices can exist without being shown in the form of **discrimination**. Similarly, **discrimination can be shown without prejudice**. Yet, the two go together very often.

Wherever prejudice and discrimination exist, conflicts are very likely to arise between groups within the same society. Our own society has witnessed many deplorable instances of discrimination, with and without prejudice, based on gender, religion, community, caste, physical handicap, and illnesses such as AIDS.

Sources of Prejudice-

- **Learning:** Prejudices can be **learned through association, reward and punishment, observing others, group or cultural norms and exposure to information that encourages prejudice**. The family, reference groups, personal experiences and the media may play a role in the learning of prejudices.
- **A strong social identity and ingroup bias:** Individuals who have a strong sense of social identity and have a very positive attitude towards their own group boost this attitude by holding negative attitudes towards other groups.
- **Scapegoating:** This is a phenomenon by which the **majority group places the blame on a minority outgroup for its own social, economic or political problems. And minority is too weak to defend itself**.
- **Kernel of truth concept:** Sometimes people may continue to hold stereotypes because they think that, after all, there must be some truth, in what everyone says.
- **Self-fulfilling prophecy:** In some cases the **target group may behave in ways that justify the prejudice, that is, confirm the negative expectations**.

STRATEGIES FOR HANDLING PREJUDICE:

The strategies for handling prejudice would be effective if they **aim at:**

- a. minimising opportunities for learning prejudices,
- b. changing such attitudes,
- c. de-emphasising a narrow social identity based on the in-group, and
- d. Discouraging the tendency towards self-fulfilling prophecy among the victims of prejudice.

These **goals can be accomplished through:**

- **Education and information dissemination**, for correcting stereotypes related to specific target groups, and tackling the problem of a strong in-group bias
- **Increasing intergroup contact** allows for direct communication, removal of mistrust between the groups, and even discovery of positive qualities in the out-group. However, these strategies are successful only if : -

->the two groups meet in a cooperative rather than competitive context,

->close interactions between the groups helps them to know each other better, and - the two groups are not different in power or status.

- **Highlighting individual identity rather than group identity**, thus weakening the importance of group as a basis of evaluating the other person.

Basis	Prejudice	Discrimination
Definition	A negative attitude or belief towards a particular group or individual based on stereotypes.	Unfair behavior or actions against a group or individual based on prejudice.



Basis	Prejudice	Discrimination
Nature	Cognitive and emotional (thoughts & feelings)	Behavioral (actions & practices)
Expression	Exists in thoughts, opinions, and feelings but may not always be openly expressed.	Evident in actions and treatment towards others.
Example	Believing that a particular gender is not good at leadership roles.	Not hiring a person because of their gender, despite their qualifications.
Scope	Can exist without discrimination.	Usually stems from prejudice but requires action.
Impact	Reinforces stereotypes and biases.	Leads to social injustice and inequality.

Relationship between Attitude and Beliefs

Aspect	Attitude	Belief
Connection	Attitudes are influenced by beliefs.	Beliefs form the foundation of attitudes.
Dependency	Cannot exist without underlying beliefs.	Can exist independently, but influence attitudes.
Example	A person has a positive attitude toward exercise because they believe it improves health.	The belief that exercise is beneficial leads to a positive attitude toward it.

Difference between Attitude and Beliefs

Basis	Attitude	Belief
Definition	A psychological tendency expressed through feelings, thoughts, and behaviors toward something.	An idea or conviction that an individual holds to be true.
Nature	Evaluative (positive, negative, or neutral)	Cognitive (based on knowledge, experience, or faith)
Formation	Developed through experiences, social influences, and emotions.	Developed through culture, upbringing, religion, and personal experiences.
Flexibility	Can change over time with new experiences or information.	More resistant to change and deeply ingrained.
Example	Having a positive attitude toward environmental conservation.	Believing that climate change is real and harmful.

THEORIES OF ATTITUDE CHANGE

Attitude change occurs when individuals modify their beliefs, emotions, or behaviors toward a particular object, person, or situation due to new experiences or information. The following theories explain how attitudes change:

1. Leon Festinger's Cognitive Dissonance Theory

Concept:

- Proposed by **Leon Festinger (1957)**, this theory states that when individuals experience **inconsistency (dissonance)** between their attitudes and behaviors, they feel psychological discomfort.
- To reduce this discomfort, they either **change their attitude, change their behavior, or justify their behavior**.



Example:

A person who smokes cigarettes **knows that smoking causes cancer**. This creates **cognitive dissonance**. To reduce this discomfort, they may:

1. **Change behavior** – Quit smoking.
2. **Change attitude** – Believe that smoking is not that harmful.
3. **Justify behavior** – Say, “Many people smoke and live long lives.”

Flowchart of Cognitive Dissonance Theory

Inconsistency between attitude & behavior



Cognitive Dissonance (Discomfort)



Motivation to reduce dissonance



Change in attitude or behavior OR Justification

Diagram: Cognitive Dissonance Model

Attitude: "Smoking is unhealthy"



Cognitive Dissonance



(To reduce discomfort)



Change Attitude Change Behavior Justify Action



"Smoking isn't that bad" Quit smoking "I exercise, so it balances out"

2. S.M. Mohsin's Two-Concept Theory

Concept:

- Indian psychologist **S.M. Mohsin** proposed that attitude change depends on **two key factors**:
 1. **Credibility of the source** – The person or medium conveying the message should be trustworthy.
 2. **Emotional appeal** – The message should create a strong emotional impact on the individual.

Example:

- A **celebrity doctor** advocating for a healthy diet is more likely to change people's attitudes because of **high credibility**.
- A **heartbreaking anti-smoking ad** may change smokers' attitudes because of **strong emotional appeal**.

Flowchart of Mohsin's Two-Concept Theory

Message is delivered



Source credibility + Emotional appeal



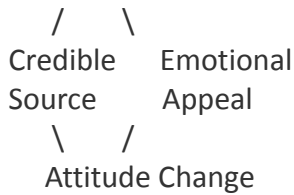
Processing of message



Change in attitude

Diagram of Mohsin's Theory

(Message)



3. P-O-X (Balance) Theory by Fritz Heider

Concept:

- This theory, developed by **Fritz Heider**, explains that people prefer balanced relationships between **Person (P)**, **Other (O)**, and **Attitude Object (X)**.
- If there is an **imbalance (inconsistency)**, people tend to change their attitudes to restore balance.

Example:

- Suppose **Person (P)** likes their friend **(O)** but their friend dislikes a movie (X) that P likes. This creates an **imbalance**.
- To restore balance, P may:
 1. Change attitude toward the movie (start disliking it).
 2. Convince the friend to like the movie.
 3. Accept the imbalance but feel discomfort.

Diagram of P-O-X Balance Theory

Balanced Relationship (No Attitude Change Required)

P (+) → O
O (+) → X
P (+) → X

Imbalanced Relationship (Attitude Change Required)

P (+) → O
O (-) → X
P (+) → X

Flowchart of P-O-X Theory

Person (P) interacts with Other (O) and Object (X)



Check if relationships are balanced



If balanced → No attitude change

If imbalanced → Change in attitude to restore balance

Summary Table

Theory	Key Concept	Example
Cognitive Dissonance Theory (Leon Festinger)	People feel discomfort when attitude and behavior are inconsistent, leading to attitude change.	A smoker who knows smoking is harmful may quit smoking to reduce discomfort.
Two-Concept Theory (S.M. Mohsin)	Attitude change depends on credibility of the source and emotional appeal.	A public health campaign by a trusted doctor changes people's views on vaccines.
P-O-X Balance Theory (Fritz Heider)	People prefer balanced relationships; imbalance leads to attitude change.	A person starts disliking a product if their best friend dislikes it.

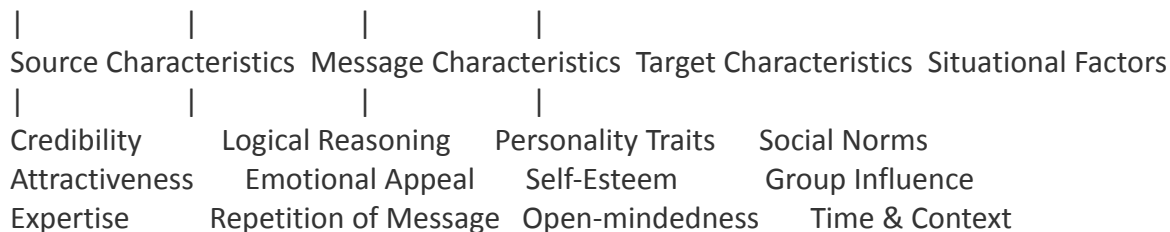
BOX 6.2 TELLING A LIE Summary of "Telling a Lie for \$20" Experiment (Festinger & Carlsmith, 1959)



This experiment by **Leon Festinger and James Carlsmith (1959)** tested the **Cognitive Dissonance Theory** by examining how people change their attitudes when forced to behave against their beliefs.

Flowchart of Factors Influencing Attitude Change

Factors Influencing Attitude Change



Explanation of Factors Influencing Attitude Change

Factor	Description	Example
Source Characteristics	The credibility, attractiveness, and expertise of the person delivering the message.	A doctor promoting a healthy lifestyle is more convincing than a layperson.
Message Characteristics	The content, logic, emotional appeal, and repetition of the message.	Advertisements use emotions (happiness, fear) to persuade people.
Target Characteristics	The personality, self-esteem, and openness of the person receiving the message.	People with high self-esteem are more resistant to attitude change.
Situational Factors	Social norms, peer pressure, and environmental context can influence attitude change.	A person may stop smoking if their friend group disapproves.

Experiment Process (Flowchart)

Participants asked to do a boring task



They were asked to lie to the next participant, saying the task was interesting



Participants were paid either \$1 or \$20 for lying



Later, participants were asked their true opinion about the task



Results:

- \$1 group changed their attitude and rated the task as interesting (High Dissonance)
- \$20 group rated it as boring (Low Dissonance, Justified Lying)

Explanation of Findings

Group	Payment	Attitude Change	Reason
\$1 Group	Given only \$1 to lie	Reported that the task was interesting	High cognitive dissonance → Justified lying by changing their attitude
\$20 Group	Given \$20 to lie	Admitted the task was boring	Low cognitive dissonance → Justified lying by external reward (money)



SOCIAL INFLUENCE AND GROUP PROCESSES

Q Define Group. Describe some features of the group.

Group: organised system of 2 or more people who interact and are interdependent, have common motives, have a set of role relationships among members and have norms that regulate the behaviour of its members.

Features of groups:

- 1- Collection of people with common goals & motives: towards a goal, away from threat
- 2- 2 or more people: perceive themselves as belonging to the group- each group is unique
- 3- Members are interdependent
- 4- Members interact with each other directly or indirectly
- 5- Members satisfy needs through joint association: influence each other
- 6- Set of norms and roles: specific functions for each member, adhere to norms on how one must behave, expected behaviour etc

CROWD	Collection of people present at a place/situation by chance.	No structure, no sense of belongingness, no interdependence, people show irrational behaviour
TEAMS	Members have complementary skills & are committed to a common goal/purpose	Members are mutually accountable, and derive +ve synergy from coordinated efforts
AUDIENCE	Collection of people assembled for a specific purpose	This is passive and could turn into a mob
MOB	Collection of people with a definite sense of purpose. Polarisation in attention and actions in common direction	Homogeneity of thought & behaviour. Mobs are impulsive

GROUPS	TEAMS
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Performance dependent on contributions of individual members	Both individual contributions & teamwork matter
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Leader/head holds responsibility	Members hold themselves responsible
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Advantages of joining groups:

- we are simultaneously members of different groups
- +different groups satisfy different needs
- +could create pressures due to competing demands & expectations
- 1- Security: groups reduce insecurity
- being with people: sense of comfort/protection
- people feel stronger: less vulnerable to threats
 - 2- Status: recognised group- gives feeling of power and importance
 - 3- Self esteem: feelings of self worth & +ve social identity
- member of prestigious group: enhances self concept
 - 4- Goal achievement: group helps to attain some goals which can't be attained alone (power in the majority)
 - 5- Provides knowledge & information: broadens views, helps supplement info
 - 6- Satisfaction of psychological & social needs: like sense of belongingness. Giving & receiving attention, love & power

Group formation:

- some form of contact & interaction between people is needed
 - 1- Proximity: closeness& repeated interactions with the same people (get to know their interests, attitudes& background)
 - 2- Similarity: people prefer consistency – consistent relationships (reinforces & validates opinions& values; feel we're right)
 - 3- Common motives & goals: groups facilitate goal attainment

Stages of group formation: given by Tuckman (IMPORTANT)

- 1- Forming: members first meet – there is uncertainty about group & goal and how it will be achieved).
- they try to get to know each other: there is excitement & apprehension
 - 2- Storming: intragroup conflict- about how the goal is to be achieved, who's the leader & who will perform what task (hierarchy of leadership & how to achieve goal is developed)
 - 3- Norming: develop normas related to group behaviour (development of +ve group identity)
 - 4- Performing: structure of the group has evolved and is accepted (towards goal achievement). At times, this is the last stage of group development
 - 5- Adjourning: once the function is over, the group may be disbanded



Group structure: - over time there is regularities in distribution of tasks, responsibilities assigned to members and status of members

Elements of group structure:

- 1- Roles: socially defined expectation that individuals in given situations are expected to fulfil
ie. Typical behaviour that depicts a person in a given social context
- role expectations: behaviour expected of someone in a particular role
- 2- Norms: (unspoken rules) expected standards of behaviour & beliefs established, agreed upon & enforced by group members
- 3- Status: relative social position given to group members by others
- ascribed (given due to ones seniority) or achieved (because of expertise or hardwork)
- member of a group: enjoy status, and want to be members of prestigious groups.
- within groups, different members have different prestige & status
- 4- Cohesiveness: togetherness, binding or mutual attraction among members
- more cohesiveness: members start thinking, feeling & acting as a social unit (no isolated individuals). There is an increased desire to remain in the group (we feeling-sense of belongingness). It is difficult to leave the group or gain membership into it
- extreme cohesiveness leads to group think and is negative.

Types of groups:

PRIMARY GROUP	SECONDARY GROUP
Pre-existing formations that are usually given to a person. Person usually remains a part of it through their lifetime	Groups which individuals join by choice
Includes face-to-face interaction & close physical proximity. Member share warm, emotional bonds	Relationships among members are more impersonal, indirect and less frequent
Central to person's functioning; major role in developing values & ideals	These may or may not be short lived
Boundaries are less permeable: cant choose membership, join or leave easily	It is easy to leave and join another group
Egs: family, religion, caste	Egs: political party

FORMAL GROUP	INFORMAL GROUP
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Functions, roles to be performed are explicitly stated	Roles of each member not so definite and specified. Close relationship among members exist
Formation based on specific rules or laws and members have definite roles. Set of norms help establish order	Formation not based on rules and laws
Egs: office, university	Egs: peer group
INGROUP	OUTGROUP
One's own group- "we" (eg- India)	Another group- "they" (eg- Pakistan)

Members in the group: similar, viewed favourably, have desirable traits	Member of outgroup: viewed differently, negatively in comparison to ingroup
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Influence of group on individual behaviour

1- Social Loafing: is the reduction in individual effort when working on a collective task

- individual performing an activity with the others as part of a larger group.
- individuals work less hard in a group than alone
- don't know how much effort each one is putting in
- presence of others leads to arousal. This motivates individuals to enhance their performance (only when a person's efforts are individually evaluated).

Causes: a) members feel less responsible for the overall task and thus exert less effort.

- b) performance of the group isn't compared with other groups
- c) motivation decreases as contributions are not individually evaluated
- d) no/improper coordination between members
- e) belonging to the same group is not important for members (it is only an aggregate of individuals)

Can be reduced by: a) making efforts of each person identifiable

- b) increasing pressure to work hard- make members committed, motivated
- c) increase apparent importance and value of task
- d) make them feel their individual contribution is important
- e) strengthen group cohesiveness: increase motivation for successful group outcome

2- Group Polarisation: IMPORTANT groups are likely to take more extreme decisions than individuals would take alone

- strengthening of group's initial position because of group interaction
- dangerous repercussions: groups may take extreme positions (very weak to very strong decisions)

Causes: a) in the company of like minded people, you're likely to hear newer arguments favouring your viewpoints

- b) Bandwagon effect: when you find others sharing your viewpoint, you feel your view is validated by the public
- c) when people have similar views as you, you're likely to perceive them as ingroup (start identifying with them, show conformity: views become strengthened)



